FOR SOLO PHYSIOTHERAPISTS, CLINICS AND HOME-REHAB PRACTICES

AI Prompt Pack for Indian Physiotherapists

& Home-Rehab Practices

80 copy-paste prompts and 7 WhatsApp scripts for the assessment, home programmes, patient conversations, operations and growth of an Indian physiotherapy practice.

80	7	14	2026-27
PROMPTS	WHATSAPP SCRIPTS	PARTS	EDITION

Facts verified November 2026. Clinical judgement stays with the treating physiotherapist.

DIGITALAIKART

AI Prompt Pack for Indian Physiotherapists & Home-Rehab Practices

2026-27 Edition

Eighty copy-paste prompts and seven WhatsApp scripts for the assessment, home exercise programmes, patient conversations, operations and growth of an Indian physiotherapy practice - built around home visits, package billing, WhatsApp-first communication and the 2026 registration and data-protection rules.

Who this is for: solo physiotherapists, two-person clinics, home-visit practices and rehab teams who do their own admin and their own marketing.

> **What this pack is not**
> It is not a diagnostic tool and not a substitute for clinical judgement. Every output is a draft for a registered physiotherapist to review before it reaches a patient. Nothing here is legal, tax or medical advice.

Contents

Contents 2

How to use this pack in the next ten minutes 3

Part 01 · The 2026 reality check **4**

1.1 Registration is now a legal requirement, not a membership 6

Part 02 · The privacy rail **8**

2.1 The nine things that must never leave your phone 10

Part 03 · The prompt formula **11**

3.1 The seven blocks 13

Part 04 · Assessment and clinical reasoning **14**

Part 05 · Home exercise programmes and adherence **20**

Part 06 · Patient communication and the money conversation **27**

Part 07 · Operations: the evening that disappears 34

Part 08 · Growth: getting found, getting referred 41

Part 09 · Team and money 48

Part 10 · Seven WhatsApp scripts, ready to send 53

Part 11 · The routines 56

11.1 The 20-minute admin day . . . 58

11.2 The weekly hour . . . 58

11.3 The monthly half-day . . . 58

Part 12 · The 30-day adoption plan 58

Part 13 · Twelve ways AI makes a physio practice worse 61

Part 14 · The prompt index 63

A last page, for the wall . . . 67

How to use this pack in the next ten minutes

This is not a book about artificial intelligence. It is a working tool for a physiotherapy practice in India: 80 prompts you paste, fill, and send, plus 7 WhatsApp scripts that already carry the tone most Indian patients respond to.

Three rules make the difference between a pack that saves you an hour a day and one that sits in your Downloads folder.

- **Rule 1 - Paste, then edit the brackets.** Every prompt has [SQUARE BRACKETS]. Replace every one with your own case facts. A prompt sent with brackets still filled in produces generic filler.
- **Rule 2 - Never paste a name, a phone number, an address or a hospital ID.** Part 2 gives you a two-minute de-identification routine and the prompt that does it. India's data protection clock is running (Part 1), and a patient's case history is the most sensitive data you hold.
- **Rule 3 - AI drafts, you decide.** No prompt in this pack asks a model to diagnose, to set a dosage, or to clear a patient for discharge. It drafts structure, language, options and checklists. The clinical judgement, the hands-on assessment and the final signature stay with you.

The order that works

Read Part 1 and Part 2 once - about fifteen minutes - because they change how you use everything after them. Then jump straight to the part that matches this week's pain. If your week is full of home visits, start at Part 5 and Part 7. If your diary has gaps, start at Part 8. If patients keep dropping out after four sessions, start at Part 5 and Part 6.

If this is your problem	Go to
Patients stop coming after the pain settles	Part 5 (adherence) and Part 6 (the continue-conversation)
Exercise sheets take 20 minutes each	Part 5, prompts 16 to 20
Enquiries ask 'kitna charge hai?' and vanish	Part 6, prompts 31 to 33
Admin eats the evening	Part 7 and the routine in Part 11
The diary has empty slots next month	Part 8, prompts 59 to 72
You are hiring your first associate	Part 9, prompts 73 to 76

What is in the pack

80	7	14	30
copy-paste prompts	WhatsApp scripts	parts, each a single job	day adoption plan

Every prompt is written for an Indian practice: home visits in traffic, WhatsApp as the primary channel, package billing in sessions, family members who attend the consultation, Hindi and Hinglish alongside English, and patients who ask whether insurance will pay.

What this pack will not do

It will not assess a patient, read an X-ray or MRI, decide a diagnosis, prescribe a dosage, or replace your clinical reasoning. Every output is a draft that a registered physiotherapist reviews before it reaches a patient. Where a prompt touches a legal or regulatory question - consent, billing, data - the prompt tells the model to flag it for your own review, not to answer it as law.

PART 1

01

The 2026 reality check

Three things changed this year that affect how you use AI, how you present your credentials, and what you can promise a patient about insurance.

1.1 Registration is now a legal requirement, not a membership

If you qualified in physiotherapy and have been practising on the strength of your degree, the ground moved in 2026. The National Commission for Allied and Healthcare Professions (NCAHP) - constituted under the NCAHP Act, 2021, which covers 57 recognised allied and healthcare categories including physiotherapy - notified the **Registration of Allied and Healthcare Professionals Regulations, 2026** on 31 August 2026, published in the Gazette of India on 15 September 2026.

What those regulations do, in plain terms:

- Registration with your State Allied and Healthcare Council becomes the licence to practise, with a lifetime Unique Identity Number carrying a QR code that a patient, an employer or a hospital can verify.
- A degree alone no longer gets you into clinical practice. Interim registration is needed to do the mandatory internship, and full registration follows physical verification of your original certificates at the council.
- Unregistered practice now carries consequences under the Act - disqualification from employment in public and private hospitals, monetary penalties, and penal liability that has been described as ranging from six months to two years of imprisonment.

Where things stand (verified November 2026)	Detail
Commission	NCAHP constituted under the NCAHP Act, 2021; 18 competency-based curricula finalised; enrolment portals live since October 2024
State councils	29 to 30 of 36 States and Union Territories have notified a State Allied and Healthcare Council; NCAHP flagged about ten as constituted with deviations from the Act
No council notified	As of NCAHP's 16 September 2026 status table: Chandigarh, Chhattisgarh, Delhi, Ladakh, Lakshadweep, Puducherry
Where you apply	To your State Council where a functional council exists; to the Central Register where it does not. Confirm with the council or NCAHP before you pay any fee
The AHPR portal	Enrolment on the ABDM Allied and Healthcare Professionals Registry is a data-collection exercise, not statutory registration under the Act
Workforce size	There is still no official count of physiotherapists in India; World Physiotherapy estimates about 52,000 practising professionals

Why this belongs in an AI prompt pack

Because two prompts later in this book - the credential explainer for patients (prompt 41) and the itemised bill note (prompt 51) - are only useful if they carry the right registration number and the right wording. The model cannot know your UID. You paste it in. That is also the honest answer to the fear that AI will replace clinicians: it cannot hold a registration.

1.2 The data protection clock is running

You handle health data every day, and from 2026 that has a legal frame with dates in it.

Date	What comes into force under the DPDP framework
11 August 2023	Digital Personal Data Protection Act, 2023 enacted as Act No. 22 of 2023
13 November 2025	DPDP Rules, 2025 notified (G.S.R. 846(E)); definitions and the provisions constituting the Data Protection Board come into force
13 November 2026	Consent Manager provisions - registration with the Board opens. Using a consent manager is not mandatory for a clinic

Date	What comes into force under the DPDP framework
13 May 2027	The main operational duties for Data Fiduciaries: notice, consent, security safeguards, breach intimation, retention and erasure, grievance redressal. MeitY has described the Act as becoming fully enforceable from this date

A solo physiotherapy practice is a Data Fiduciary for its patient records. You do not need a compliance department. You do need to know what data you hold, who else sees it, and what leaves your phone - including what you paste into a chatbot. Part 2 is the practical half of this: a two-minute routine that keeps patient identity out of any model, so the question never arises.

1.3 AI in a physio practice: what it is actually good at

The honest split, after using these tools on real practice work:

AI is genuinely good at	AI is unreliable at, and must not be used for
Turning your messy notes into a clean, structured record	Diagnosing a patient, or ranking diagnoses as if it were an assessment
Drafting a home exercise programme's structure, dosage table and progression rules for you to check	Deciding a load, a dosage or a progression for a patient you have not assessed
Rewriting clinical language into language a family understands	Reading an X-ray, MRI or report and telling a patient what it means
Producing options, comparisons and checklists you then verify	Citing a guideline, a study or a clause number that it may have invented
Writing the first draft of a proposal, a review reply, a referral letter, a job description	Promising insurance reimbursement, or stating what a policy covers

> **The one failure that costs you a patient**
> A confident, fluent, wrong answer. Models do not signal uncertainty the way a colleague does. Every prompt in this book therefore ends with a verification instruction, and Part 13 lists the twelve failure modes to watch. If an output contains a number you did not supply - a session count, a percentage, a price, a study - treat it as a typo you have not found yet.

1.4 The insurance reality, so you stop over-promising

Patients ask 'insurance se ho jayega?' and the wrong answer creates a fight at the counter three weeks later. What Indian health policies actually do, as of November 2026:

- Physiotherapy is usually reimbursed inside the **pre-hospitalisation** window (commonly 30 to 60 days) or the **post-hospitalisation** window (commonly 60 to 180 days), and only when it is medically necessary, advised by the treating doctor, and directly related to the event that caused the hospitalisation.
- Day-care and domiciliary hospitalisation may include physiotherapy, but only if your patient's own policy wording, schedule and endorsements say so - and domiciliary cover normally requires that hospital-level treatment was needed and that the patient could not be moved or no bed was available.
- Standalone out-patient clinic visits, with no hospitalisation behind them, are generally not covered. IRDAI expects insurers to offer products, add-ons and riders across situations including OPD, day care and home care, but no single policy has to include every benefit.
- The claim is judged on the policy wording, not on the label on the form. 'Domiciliary' written by a hospital does not make a claim payable.

So the safe sentence, and the one prompt 40 helps you phrase for a specific patient, is: *physiotherapy is usually covered only when it is part of a hospitalisation episode - check your policy's pre- and post-hospitalisation clauses before you plan around reimbursement.*

1.5 The tool stack, with November 2026 prices

You do not need all three. Most solo practices do fine on one paid subscription and the free tier of another. Prices below are the India-facing subscription prices reported in the first week of November 2026 - confirm on the provider's own pricing page before you pay, because these move.

Tool	Current top models (November 2026)	India pricing to check	Best use in a physio practice
ChatGPT (OpenAI)	GPT-6 Sol on paid plans, GPT-6 Luna on Free and Go, GPT-6 Astra as the top reasoning setting	Go about Rs 399/month; Plus about Rs 1,999/month; Pro from about Rs 10,699/month	All-round: notes, HEP drafting, WhatsApp copy, images of exercise positions
Gemini (Google)	Gemini 4 Argon on AI Pro and above; the free tier runs on Flash-Lite	AI Plus about Rs 399/month; AI Pro about Rs 1,950/month	Long documents, PDFs of surgeon protocols, Workspace users
Claude (Anthropic)	Fable 5.1 for the hardest work, Opus 5.5 as the recommended default, Sonnet 5.5 and Haiku 5.5 for speed	Pro about Rs 2,399/month	Long-form writing: patient handouts, proposals, referral letters

A free tier is enough to test every prompt in this pack. Upgrade only when you find yourself hitting limits twice a week on the same kind of task.

PART 2

02

The privacy rail

Two minutes of routine that keeps patient identity out of any AI tool - and the prompt that does it for you.

IN THIS PART

01 De-identify a case note before you paste it

02 Leak check: audit what you are about to paste

2.1 The nine things that must never leave your phone

De-identification is not about hiding the injury. It is about removing the combination of details that makes a person identifiable. Strip these nine, in this order:

- ☐ Name - patient, family, referring doctor, hospital, employer
- ☐ Phone numbers, WhatsApp numbers, email addresses, UHID or hospital registration numbers
- ☐ Exact address - keep the city and the broad area only ('Pune, Kothrud' becomes 'a Tier-1 city, residential area')
- ☐ Dates of birth and exact ages under 90 - use a band ('mid-50s')
- ☐ Exact dates of injury, surgery or admission - use intervals ('about 6 weeks post-op')
- ☐ Photographs, screenshots of reports, and file names that carry a name or a number
- ☐ Occupation when it is unusual enough to identify ('the only physiotherapist-referred badminton coach in the district' becomes 'a racket-sport coach')
- ☐ Rare diagnoses combined with a small town - widen the place, or generalise the condition
- ☐ Any detail a neighbour would recognise - the building, the school, the wedding, the accident that made the news

2.2 The routine

Write your case facts in the model's own words, in this shape, every time. It takes about ninety seconds once it is a habit, and it produces better output anyway because the model gets a clean structured brief instead of a paragraph of storytelling.

PROMPT 01

De-identify a case note before you paste it

Use when: Use this the first three or four times, until the routine is automatic. Paste your raw note in place of the bracket.

```
You are helping me prepare a de-identified case brief for an AI assistant.

Here is my raw clinical note:
[PASTE RAW NOTE - or type it in shorthand]

Rewrite it as a de-identified brief with these sections:
1. Age band and sex (e.g. "female, early 60s")
2. City tier and setting (clinic / home visit / tele), NOT the address
3. Presenting problem in clinical terms
4. Onset and duration as intervals ("about 6 weeks ago"), not dates
5. Relevant history, comorbidities, medication categories (not brand names)
6. Aggravating and easing factors
7. Baseline objective findings I recorded (keep every number I gave you)
8. My working hypothesis, labelled as MY hypothesis
9. What I need help with today, in one line

Rules:
- Remove every name, phone number, address, date, ID number and file name.
- Do not add any clinical detail I did not give you.
- Do not suggest a diagnosis in this step.
- End by listing anything you removed, so I can check nothing identifying slipped through.
```

Why it works: The last line is the useful part: it makes the model show its work on the privacy step, and you get a second look at what it thought was identifying.

PROMPT 02

Leak check: audit what you are about to paste

Use when: Run this on a brief you prepared yourself, before pasting it anywhere. Ten seconds, and it catches the detail you forgot.

```
Act as a privacy reviewer for an Indian healthcare practice preparing text for an AI tool.

Read the text below and list:
1. Every element that could identify the person, directly or in combination.
2. For each, the safer replacement.
3. A one-line verdict: SAFE TO PASTE, or FIX THESE FIRST.

Assume the reader may know the patient, the family, or the neighbourhood.

Text:
[PASTE YOUR BRIEF]
```

Why it works: Two identifiers that look harmless together can identify a person: a rare condition plus a small town, or a date of surgery plus a named surgeon.

2.3 If you have already pasted something

Do not panic, and do not delete your records. Do this instead: open the conversation settings and delete that chat; if the tool offers a setting that keeps chats out of training, switch it on for the future; note the date and what was pasted in your own register; and if the exposure involved a large amount of identifiable health data, take advice before deciding whether it needs to be treated as a breach. The DPDP duties on breach intimation for Data Fiduciaries are among the provisions scheduled for 13 May 2027, which is exactly the runway you have to build the habit now.

The one-line habit

Name, number, address, date: never in, always out. If a prompt output is only useful because it quotes the patient's name, the prompt was doing the wrong job.

PART 3

03

The prompt formula

Seven blocks that turn a vague request into an output you can actually use - and the difference between the two, side by side.

IN THIS PART

03 The prompt fixer - upgrade any request you already type

3.1 The seven blocks

Block	What it does	Example
1. ROLE	Sets the standard the model writes to	You are a practice manager writing for an Indian physiotherapy clinic
2. CASE OR BUSINESS FACTS	The only facts the model is allowed to use	De-identified brief / your session price / your area / your team size
3. TASK	One job, stated as an instruction	Draft a 4-week home programme structure with dosage columns
4. CONSTRAINTS	Length, language, tone, reading level, what to avoid	Under 250 words, simple English, no medical jargon, no promises of cure
5. OUTPUT SHAPE	The exact format you want to receive	A table with columns Week, Exercise, Sets x Reps, Frequency, Progression rule
6. SAFETY LINE	Stops the model inventing clinical content	Do not add exercises I did not list. Do not state a diagnosis. Insert [CHECK] wherever I must verify
7. VERIFY LINE	Tells you what to check before use	End with a three-item list of things I must confirm against my own assessment

Most people write block 3 and stop. That is why most AI output feels like a template: with no role, no constraints and no output shape, the model defaults to the average of everything it has read.

3.2 The same request, twice

Weak (what most people type)

"Write an exercise plan for knee pain." - No role, no patient, no constraints, no shape, no safety line. Output: a generic list of quadriceps sets and straight leg raises, the same for a 24-year-old footballer and a 68-year-old with arthritis.

Strong (blocks 1 to 7)

"You are a musculoskeletal physiotherapist drafting a home programme STRUCTURE for me to review. Patient: female, early 60s, bilateral knee OA, home visit setting, no gym equipment, has a chair and a bed, walks to the temple daily. Task: draft a 4-week progression structure. Constraints: 4 exercises maximum, all doable on a chair or bed, simple English for a print handout, no promise of cure. Output: table with Week, Exercise, Sets x Reps, Hold, Frequency, Progression rule. Do not add exercises beyond the four categories I list: quadriceps, hip abductor, hamstring, balance. Insert [CHECK] where I must confirm load. End with three things I must verify against my assessment."

Same model, same day, completely different output - because the second request told it what a good answer looks like.

3.3 Make any prompt yours in three edits

- **Edit the setting.** Clinic, home visit or tele changes everything: equipment, travel time, who is in the room, whether a caregiver is present.
- **Edit the language.** Most of your patients read English but think in Hindi, Marathi, Tamil, Bengali or Telugu. Prompts 18 and 29 give you Hinglish and Hindi versions; the pattern works for any language pair - ask for the message in simple English plus a version in your patient's language in the same script.
- **Edit the constraint that keeps biting you.** If your patients stop at session four, add 'write it so a patient who feels 70 per cent better still sees why to continue'. If your patients are elderly, add 'assume the reader is 70 and reading on a phone without glasses'.

PROMPT 03

The prompt fixer - upgrade any request you already type

Use when: Use this whenever you have a rough request in your head. It converts it into a seven-block prompt in one step.

```
You are a prompt engineer for a physiotherapy practice in India.

I will give you a rough request. Rewrite it as a complete, reusable prompt with these seven blocks,
labelled:
ROLE / FACTS I WILL PASTE / TASK / CONSTRAINTS / OUTPUT SHAPE / SAFETY LINE / VERIFY LINE

Rules for the rewrite:
- The safety line must forbid inventing clinical detail, dosages, diagnoses, guideline citations or
statistics.
- The verify line must ask for a short list of what a registered physiotherapist must check before use.
- Keep it under 200 words so I will actually paste it daily.
- Use plain English, no jargon.

My rough request: [WHAT YOU WANT, IN YOUR OWN WORDS]
```

Why it works: Save the output as a note on your phone with a short title. Twelve saved prompts will cover most of your week.

PART 4

04

Assessment and clinical reasoning

Twelve prompts that structure your thinking and your record - and never make the diagnosis for you.

IN THIS PART

04 Turn a messy note into a structured subjective history
05 Build my hypothesis list - and argue against each one
06 Choose the right outcome measure for this patient
07 The red-flag screen I must not skip
08 Shortlist special tests I can actually perform here
09 A baseline objective record I can repeat at every visit
10 Reassessment: is this progress, a plateau, or a wrong plan?
11 Discharge readiness check

and 4 more in this part

Read this part with the safety line in mind: these prompts organise what you already found. They do not find it for you. Every output is a draft of your reasoning, not a second opinion from a machine.

PROMPT 04

Turn a messy note into a structured subjective history

Use when: Immediately after a first assessment, when the note is shorthand and you still remember the story.

```
You are a physiotherapist's documentation assistant in India.

Below is my shorthand from a first assessment. Rewrite it as a structured subjective history with these
headings:
Chief complaint / Onset and course / Aggravating factors / Easing factors / 24-hour behaviour /
Functional impact (work, sleep, self-care, walking) / Previous episodes and treatment / Comorbidities
and medication categories / Patient's own goal in their words.

Rules:
- Use only the facts below. Where a heading has no information, write "Not recorded".
- Do not infer, do not add findings, do not suggest a diagnosis.
- Keep the patient's own phrases in quotes where I wrote them.
- British/Indian clinical English, concise, no filler.

My shorthand:
[PASTE DE-IDENTIFIED SHORTHAND]
```

Why it works: The 'Not recorded' instruction is worth more than the rest of the prompt: it shows you the gaps in your own history-taking, which is where most missed red flags live.

PROMPT 05

Build my hypothesis list - and argue against each one

Use when: When you have findings but the picture is muddled and you want your differential tested before you commit to a plan.

```
Act as a senior musculoskeletal physiotherapist reviewing my reasoning - not as a diagnostician.

Here is my de-identified case brief:
[PASTE BRIEF FROM PROMPT 01]

Here is my working hypothesis: [YOUR HYPOTHESIS]
Here is the evidence for it from my findings: [LIST]

Do this:
1. List up to three alternative explanations a careful clinician would keep open at this stage, each in
one line, with the specific finding that would support it.
2. For each alternative, name the ONE test, question or measure that would best discriminate it in my
setting (clinic / home visit).
3. Tell me what in my findings does NOT fit my working hypothesis.
4. List any red flag that my brief does not explicitly rule out.

Rules:
- Do not diagnose. Frame everything as "consider" and "would support".
- Do not invent findings. If something is missing, ask for it.
- End with one line: "This is a reasoning aid, not a diagnosis."
```

Why it works: Point 3 is the one people skip. A hypothesis you cannot attack is a habit, not a diagnosis.

PROMPT 06

Choose the right outcome measure for this patient

Use when: At baseline and at discharge, so your progress claims are measurable instead of felt.

```
You are a physiotherapy outcome-measure advisor for an Indian out-patient and home-visit setting.

Case: [PASTE DE-IDENTIFIED BRIEF]
Setting: [clinic / home visit / tele]
Time available per session: [minutes]
```

```
What I need to show: [progress to the patient / progress to a referring doctor / my own record]

Recommend:
1. One condition-specific measure and one generic measure, with the reason for each in one line.
2. The scoring range and what a meaningful change is usually taken to be - state clearly that I must confirm this against the current published source.
3. Whether it is free to use and where the form is normally obtained.
4. A 30-second way to explain the score to a patient who reads simple English.

Rules:
- Do not invent cut-off values, MCIDs or licence terms. Where you are not certain, write [VERIFY against the current source] instead of a number.
- Prefer measures that work without equipment I do not have.
```

PROMPT 07

The red-flag screen I must not skip

Use when: Before you treat, and again if the patient's story changes mid-course.

```
Act as a clinical safety checklist for a physiotherapist in India.

Condition area: [e.g. low back, neck, shoulder, knee, ankle]
Patient context: [age band, sex, relevant history, setting]

Produce a screening checklist with three columns:
| Red flag to ask about or look for | Why it matters (one line) | What I do if present (refer, urgent, or monitor) |

Rules:
- Include the serious pathologies that are commonly missed in this region and age group.
- Keep it to a single page I can keep beside me.
- Do not give me prevalence percentages or cite studies - I will verify against current guidance myself.
- End with: "This checklist supports, and does not replace, my own assessment and my obligation to refer when concerned."
```

PROMPT 08

Shortlist special tests I can actually perform here

Use when: When the setting limits you - a living room, a bed, no plinth.

```
You are a musculoskeletal physiotherapy tutor.

Region under assessment: [e.g. shoulder]
My setting and equipment: [e.g. home visit, patient's bed and one chair, no plinth]
My working hypothesis: [YOUR HYPOTHESIS]

Give me:
1. Up to five special tests or physical examination procedures that are feasible in this setting, with the position needed and the time each takes.
2. For each, what a positive finding would and would not tell me - one line each.
3. Two tests that would be useful but are not feasible here, and the cheapest substitute.

Rules:
- Do not invent sensitivity or specificity figures. If a number would help, write [VERIFY against current source].
- Flag any test that is risky or inappropriate for this patient's age or condition.
```

PROMPT 09

A baseline objective record I can repeat at every visit

Use when: First visit, so that visit six has something to compare against.

```
You are designing a one-page baseline objective record for a physiotherapy practice in India.

Case: [PASTE DE-IDENTIFIED BRIEF]
```

```
Setting: [clinic / home visit]
Equipment available: [list honestly]

Produce a fill-in table with columns: Measure | How I take it (one line) | Unit | Today's value | Re-test at (week) | Meaningful change to look for.

Rules:
- Six measures maximum. Choose the ones that will actually change my plan.
- Every measure must be repeatable by me, in the same way, at every visit.
- Include one functional task the patient does in their own home.
- No numbers in the "meaningful change" column - write [VERIFY] and I will fill it from current guidance.
```

PROMPT 10

Reassessment: is this progress, a plateau, or a wrong plan?

Use when: Every fourth or fifth session, when you are deciding whether to progress, hold, or refer.

```
Act as a clinical reviewer for my own case. Be blunt.

Baseline: [MEASURES AND VALUES FROM PROMPT 09]
Today, session [N]: [MEASURES AND VALUES]
Patient's own words about change: "[QUOTE]"
What I have done so far: [INTERVENTIONS, FREQUENCY]
My plan if nothing changes: [YOUR PLAN]

Answer in this order:
1. Which measures moved, by how much, and whether that is inside the range you would call real change (state [VERIFY] instead of inventing thresholds).
2. Which measures did not move, and the three most likely reasons - including "my plan is wrong" and "adherence is the problem".
3. One question I should ask the patient at the next visit that I probably have not asked.
4. A recommendation: continue, change one variable, or refer. Give the single strongest reason.

Rules:
- Do not flatter me. If the data is thin, say the data is thin.
```

PROMPT 11

Discharge readiness check

Use when: Before you close a case - so discharge is a decision with criteria, not a date that arrived.

```
You are a physiotherapy discharge reviewer.

Case: [PASTE DE-IDENTIFIED BRIEF]
Baseline and latest measures: [PASTE]
Patient's goal: [THEIR WORDS]
Sessions used: [N]

Produce:
1. A discharge criteria checklist for this specific case - five items, each measurable, each something I can test in one session.
2. Which criteria are already met on my data, and which are not.
3. A relapse-prevention plan the patient can follow alone: three things to keep doing, two warning signs, and when to come back.
4. A one-paragraph discharge summary I can hand over, written in simple English.

Rules:
- Do not declare the patient fit for discharge. I decide that.
- Do not add criteria that need equipment I do not have.
```

PROMPT 12

Home-visit feasibility and environment check

Use when: Before the first home visit, so the first session is treatment and not logistics.

```
You are an experienced home-visit physiotherapist in India.

Patient context: [age band, condition, mobility, who is at home during the day]
Home: [floor, lift or stairs, room available, bed type, chair type, fan/AC, floor surface]

Give me:
1. A five-question phone checklist to confirm before I travel, so the visit is not wasted.
2. What I can and cannot do in this environment, in two short lists.
3. Equipment substitutions for a plinth, a resistance band, weights and a mirror.
4. The three safety risks in this specific home, and how I raise each with the family without alarming
them.
5. A 90-second opening script for the first visit, in simple English, that sets expectations on sessions,
payment and family involvement.

Rules:
- Assume Indian traffic: build in travel reality, not textbook timings.
- Do not give medical advice about the home's suitability beyond physiotherapy safety.
```

PROMPT 13

Adapt the same assessment for an older or a younger patient

Use when: When your standard assessment does not fit the person in front of you.

```
You are a physiotherapist adapting an assessment for a different age group.

Condition: [e.g. shoulder pain]
My standard assessment for an adult of working age: [LIST WHAT YOU NORMALLY DO]
The patient in front of me: [e.g. 78-year-old, lives with son's family, hard of hearing, walks with a
stick, frightened of falling]

Rewrite my assessment for this patient:
1. What to keep, what to simplify, what to drop, and why - one line each.
2. How to ask each question so it is understood (plain words, one idea per sentence).
3. How to involve the family member present without letting them answer for the patient.
4. Two things I might over-interpret in this age group, and the check that prevents it.

Rules:
- Do not change the clinical purpose of the assessment.
- No dosage or treatment suggestions in this output.
```

PROMPT 14

The referral note a doctor will actually read

Use when: When you need to send a patient back to the orthopaedic surgeon, the physician or the neurologist.

```
You are a physiotherapist writing to a referring doctor in India.

Patient: [DE-IDENTIFIED: age band, sex, condition, weeks since onset]
What I found: [KEY FINDINGS, MEASURES AND VALUES]
What I have done: [INTERVENTIONS, SESSIONS, RESPONSE]
Why I am writing: [e.g. no progress after 6 sessions, new neurological sign, pain not settling]

Draft a one-page referral note with these sections:
Reason for referral / Relevant history in three lines / Objective findings with values / Treatment given
and response / What I am asking for / My contact details placeholder.

Rules:
- Under 250 words. No adjectives about how hard I tried.
- State facts and values only. No diagnosis beyond what I have written.
- Leave [DOCTOR NAME], [CLINIC], [REGISTRATION NUMBER] as placeholders.
```

```
- End with a single line asking for their opinion by a named route (call, WhatsApp, OPD review).
```

PROMPT 15

The 60-second case summary for a teleconsultation

Use when: When a patient or a family member calls and you cannot see the file in front of you.

```
You are my case-summary assistant.

Here is my record extract:
[PASTE DE-IDENTIFIED NOTES FROM LAST TWO VISITS]

Produce a 60-second spoken summary I can read while the patient is on the phone:
1. Who this is, in one line (age band, condition, stage).
2. Where we are against the plan (sessions done, what changed, what is pending).
3. The three questions I must ask on this call.
4. The one thing I must not forget to say (safety, payment, or next appointment).

Rules:
- Spoken English, short sentences, no clinical jargon that I would have to translate on the fly.
- Under 120 words total.
- No new clinical advice.
```

PART 5

05

Home exercise programmes and adherence

Fourteen prompts for the part of the job that eats the evening - and the part that decides whether the patient comes back.

IN THIS PART

16 A four-week programme with dosage and progression rules
17 The WhatsApp version - short enough to be read
18 The Hinglish version your patients will actually read
19 The one-page print handout
20 Progression and regression rules, written down once
21 A seven-day adherence check-in sequence
22 The patient says 'it hurts more after exercise'
23 No equipment? Substitute without losing the point

and 6 more in this part

The adherence gap is not a patient problem, it is a design problem. A programme with no dosage, no progression rule and no way to report back gives the patient nothing to adhere to. These prompts fix the design, not the patient.

5.1 Build the programme

PROMPT 16

A four-week programme with dosage and progression rules

Use when: The core prompt of this pack. Use it for every patient who needs a home programme.

```
You are a musculoskeletal physiotherapist drafting a home exercise programme STRUCTURE for me to review
and sign.

Patient: [DE-IDENTIFIED: age band, sex, condition, stage, functional goal]
Setting: [home visit / clinic / tele]
Equipment at home: [be honest: chair, bed, wall, towel, one band, staircase]
Time the patient will actually give: [e.g. 12 minutes, twice a day]
Constraints: [e.g. cannot kneel, cannot lie flat, fear of falling, works 10-hour shifts]

Produce:
1. A 4-week table: Week | Exercise | Sets x Reps (or time) | Hold | Frequency | Progression rule for next
week.
2. Four exercises maximum. Each must be doable with the equipment listed.
3. One line per exercise on how the patient knows they are doing it right.
4. A "stop and call me" list: three symptoms that mean stop, in simple words.
5. A one-line daily tracker the patient can tick on paper or reply to on WhatsApp.

Rules:
- Do not add exercises I did not ask for.
- Do not state loads, weights or progressions as fact - mark every load as [CHECK] so I set it after
assessment.
- Simple English at a Class 8 reading level. No anatomy names unless I give them.
- End with three things I must verify before this reaches the patient.
```

PROMPT 17

The WhatsApp version - short enough to be read

Use when: When the printed sheet stays on the fridge and the phone is what the patient actually looks at.

```
Rewrite the programme below for WhatsApp.

Programme: [PASTE THE TABLE FROM PROMPT 16]

Rules:
- One message, under 900 characters, no table, no PDF.
- Number the exercises 1 to 4 with an emoji-free bullet each.
- Each line: exercise name in plain words + sets x reps + how many times a day + the one cue that
matters.
- Add a final line: "Reply 1 if done, 2 if it hurt, 3 if you could not do it."
- No medical jargon. No promise of a timeline for cure.

Output: only the message text, ready to copy.
```

PROMPT 18

The Hinglish version your patients will actually read

Use when: For the majority of Indian patients who read English letters but think in Hindi.

```
Convert the programme below into natural Hinglish - Hindi in Roman letters, the way people actually type
on WhatsApp.

Programme: [PASTE FROM PROMPT 16 OR 17]

Rules:
```

```
- Keep exercise names in English where that is how patients say them (knee extension, wall slide).
- Instructions in simple Hinglish: "din mein 2 baar, 10-10 baar", "dard ho to ruk jaiye".
- Do not translate medical terms into Sanskritised Hindi. Nobody says that.
- Keep it under 800 characters.
- End with the same reply code: 1 done, 2 hurt, 3 could not do.

Output: only the message text.
```

PROMPT 19

The one-page print handout

Use when: For the patient who wants paper, and for the family member who will supervise.

```
Design a one-page A4 print handout for this programme.

Programme: [PASTE FROM PROMPT 16]
Clinic name and registration number placeholders: [CLINIC NAME], [REGISTRATION / UID]

Structure:
- Title: what this programme is for, in the patient's words.
- Four numbered exercises, each with: name, how to do it (3 short steps), sets x reps, frequency.
- A box: "Stop and call the clinic if..." with three symptoms.
- A 7-day tick grid: Day 1 to Day 7, two columns (morning / evening).
- Footer: clinic name, phone placeholder, registration number, "review on [DATE]".

Rules:
- Reading level: Class 8. Sentences under 12 words.
- Leave a blank line beside each exercise for me to write the load by hand.
- No images described, no anatomy diagrams - I will add my own photos.
```

PROMPT 20

Progression and regression rules, written down once

Use when: So the patient's next four weeks do not depend on remembering what you said at the door.

```
You are writing progression rules for a home programme.

Programme: [PASTE]
Patient's condition and stage: [BRIEF]

Produce a two-column rule set:
- "Move up when..." - three criteria the patient can judge themselves.
- "Move down when..." - three criteria.
Then a simple ladder for each exercise: Level 1 / Level 2 / Level 3, described in words the patient can
follow without equipment changes.

Rules:
- Criteria must be observable, not felt: "you can do 2 sets of 10 without stopping" beats "you feel
stronger".
- Do not prescribe loads or resistance values - mark [CHECK].
- End with: "These rules are for the patient to follow between reviews; I confirm every change at the
next session."
```

5.2 Keep them doing it

PROMPT 21

A seven-day adherence check-in sequence

Use when: Set it once per patient, then follow the replies. This is the highest-return prompt in the pack.

```
You are designing a WhatsApp adherence sequence for a physiotherapy practice in India.

Patient: [age band, condition, programme, family support at home]
Channel: WhatsApp, from the clinic's business number
```

```
Write 5 messages across 7 days:
- Day 1 evening: confirm they got the programme
- Day 2: one-question check
- Day 4: the hardest exercise, addressed directly
- Day 6: progress framing + next appointment
- Day 7: a single-line summary I can log in the file

Rules for every message:
- Under 300 characters.
- One question or one instruction, never both.
- Numeric reply codes so a one-digit answer is enough.
- No guilt, no "why did you not do it".
- Simple English; add a Hinglish variant in brackets for messages 2 and 4.

Output: a table with Day | Message | Reply codes | What I do with each reply.
```

PROMPT 22

The patient says 'it hurts more after exercise'

Use when: The most common WhatsApp message you get, and the one most likely to end a course of treatment.

```
A patient on a home programme has messaged: "[PASTE THEIR EXACT MESSAGE]"

Context: [condition, stage, what I prescribed, how long ago]

Give me:
1. A short reply in simple English (under 300 characters) that does not dismiss the pain and does not alarm them.
2. Three questions I need answered before I change anything, phrased for WhatsApp.
3. A decision tree for me: what I do if the reply is A (soreness that settles in 24 hours), B (pain during exercise that settles within an hour), C (pain that lasts into the next day), D (new symptom).
4. The one sentence I should add to my discharge advice so this message is less likely next time.

Rules:
- Do not diagnose over WhatsApp and do not tell the patient to stop treatment.
- No dosages. Where a clinical decision is needed, write [MY CALL].
```

PROMPT 23

No equipment? Substitute without losing the point

Use when: For the patient in a one-room home, a hostel, or a hotel on work travel.

```
You are a physiotherapist working with limited equipment.

Exercise I prescribed: [NAME AND PURPOSE]
Equipment I assumed: [e.g. resistance band, dumbbell, plinth]
What the patient actually has: [e.g. towel, water bottles, a wall, a bed, a staircase]

Give me:
1. Three substitutions, ranked by how close they come to the original purpose.
2. For each: how to do it, what to watch for, and what it does NOT train.
3. One warning: where a substitution changes the exercise enough that I should not use it.

Rules:
- Keep the purpose constant, change the tool.
- No invented loads. Mark [CHECK].
- Simple English.
```

PROMPT 24

The bed-and-chair programme for a frail or post-operative patient

Use when: Home visits where the patient cannot get to the floor and should not try.

```
You are a physiotherapist designing a programme for a frail patient at home.

Patient: [age band, condition, surgery or illness, mobility, supervision available]
Environment: [bed type, chair with arms or without, toilet access, stairs]

Produce a programme that:
1. Can be done entirely from a chair or the edge of a bed.
2. Has a seated version and a supported-standing version of each item.
3. Includes one breathing item and one circulation item.
4. Includes a transfer practice item with the exact words the caregiver should use.
5. Has a two-line safety statement for the caregiver, in simple language.

Rules:
- Assume a fall risk. Say so where it matters.
- No equipment beyond a chair, a bed, a towel and a wall.
- Mark every load and every progression as [CHECK].
```

PROMPT 25

Return-to-sport and return-to-work criteria

Use when: For the patient who asks 'kab tak ground pe wapas?' and wants a date.

```
You are a sports and occupational rehabilitation physiotherapist in India.

Patient: [age band, sport or job, level, condition, weeks since injury or surgery]
What I can test: [list your available tests and space]

Give me:
1. A staged return ladder (4 to 6 stages) with the criterion to enter each stage, not a timeline in
weeks.
2. The two criteria patients most often skip, and why skipping them causes recurrence.
3. A written clearance template with a placeholder for my signature and registration number.
4. A one-paragraph explanation for the patient's coach or employer, in plain language.

Rules:
- No invented percentages, no "90 per cent return to sport" claims.
- No week-by-week promises - criteria, not calendars.
- State clearly that the clearance is mine to give, not the model's.
```

PROMPT 26

The desk-worker micro-break plan

Use when: The largest, most grateful patient group in every Indian city - and the easiest to keep.

```
You are an ergonomics-aware physiotherapist.

Patient: [age band, job type, hours at a desk, screen time, commute, main complaint]
Workplace constraints: [open office / work from home / shared desk / no adjustable chair]

Produce:
1. A three-move micro-break routine, 90 seconds total, that needs no equipment and no leaving the desk.
2. When to do it: three specific triggers tied to their day, not "every hour".
3. A desk set-up checklist of six items, in order of how much each one matters for this patient.
4. A 10-line WhatsApp message for the patient, plus a Hinglish variant.
5. One line for the patient's manager, if they ask for a work-from-home note.

Rules:
- No product recommendations by brand.
- No claims that this prevents or cures anything.
- Everything must be doable in office clothes.
```

PROMPT 27

The caregiver training sheet

Use when: When the family, not the patient, will do the work between visits.

```
You are training a family caregiver, not a clinician.

Patient: [age band, condition, what they can and cannot do alone]
Caregiver: [relationship, time available, literacy, language]
Tasks to hand over: [e.g. transfers, walking practice, exercise supervision]

Produce:
1. A one-page task sheet: task | how often | how to do it in three steps | what to stop for.
2. The exact words to use when the patient refuses - three options, respectful, not bossy.
3. Three things the caregiver must never do, with the reason in one line each.
4. A daily two-line log the caregiver can fill in 20 seconds.
5. A note on when to call me, phrased so the caregiver does not hesitate.

Rules:
- Class 6 reading level. No medical terms without a plain-word explanation in brackets.
- Do not ask the caregiver to make clinical decisions.
- Include a Hinglish variant of the task sheet headings.
```

PROMPT 28

Explain a post-operative protocol without rewriting it

Use when: When the surgeon's protocol governs and your job is to translate, not to invent.

```
You are helping me explain a surgeon's post-operative protocol to a patient.

The protocol as given to me: [PASTE THE SURGEON'S PROTOCOL, OR THE RELEVANT LINES]
Patient: [age band, surgery date as weeks ago, home setting, family support]

Produce:
1. A plain-English explanation of what the protocol allows this week, in five short lines.
2. A "this week / next week" table drawn ONLY from the protocol text.
3. Three questions the patient will ask, with answers that stay inside the protocol.
4. A clear line for the patient: "If your surgeon's advice differs from this, your surgeon's advice
wins."

Rules:
- Do not add, extend or relax any restriction in the protocol.
- Where the protocol is silent, write [ASK THE SURGEON] - do not fill the gap yourself.
- No timelines the protocol does not contain.
```

PROMPT 29

The programme in Hindi, in Devanagari

Use when: For the patient or caregiver who reads Hindi but not English - a large share of home-visit families.

```
Translate this home programme into simple Hindi in Devanagari script.

Programme: [PASTE FROM PROMPT 16]

Rules:
- Everyday spoken Hindi, not Sanskritised official Hindi. Words like "dard", "ghutna", "kasrat" are
fine.
- Keep numbers as numerals (2 baar, 10 baar) for clarity.
- Keep each instruction to one line.
- Keep the reply code at the end: 1 = kiya, 2 = dard hua, 3 = nahi kar paya.
- Do not translate exercise names that patients already know in English - write them in Devanagari as
they are pronounced.

Output: only the Hindi text, ready to send.
```

PART 6

06

Patient communication and the money conversation

Fifteen prompts for the two moments that decide whether a course of treatment happens: explaining, and asking to be paid.

IN THIS PART

30 Explain the problem in three layers
31 Answer 'how many sessions and how much will it cost?' honestly
32 Three-tier package proposal
33 Objection: 'it is too expensive' / 'I will do the exercises at home'
34 The patient arrives with a doctor's note: '5 sittings only'
35 Why continue when the pain is already gone
36 The discharge summary a patient keeps
37 Brief the family without briefing them against the patient

and 7 more in this part

Physiotherapy in India is usually paid out of pocket, so the patient is weighing your fee against a hundred other claims on the family's money. Clarity is the service. Vagueness is what loses them.

PROMPT 30

Explain the problem in three layers

Use when: Every first consultation, after the assessment, before the plan.

```
You are helping me explain a diagnosis to a patient in simple language.

Diagnosis or working explanation: [AS YOU WOULD SAY IT TO A COLLEAGUE]
Patient: [age band, occupation, education level, language, who else is in the room]
What frightened them most: [e.g. "will I need surgery", "will I be able to climb stairs"]

Give me three layers, in this order:
1. ONE sentence - what is happening, in the patient's own vocabulary.
2. THREE sentences - why it happened and what usually changes it.
3. A one-line expectation setter - what physiotherapy can and cannot do here, honestly.

Then give the same three layers as a WhatsApp message under 500 characters, and a Hinglish version.

Rules:
- No medical jargon without a plain-word gloss in brackets.
- No promise of a cure and no timeline in weeks.
- No percentages or success rates.
```

PROMPT 31

Answer 'how many sessions and how much will it cost?' honestly

Use when: The single most important message you send. Get it wrong and you either lose the patient or lose money.

```
You are a practice owner in India drafting a fee-and-plan message.

My details: consultation fee [AMOUNT], per-session fee [AMOUNT], home-visit fee [AMOUNT] including
travel within [AREA], package of [N] sessions at [AMOUNT], typical course for this condition [N]
sessions, payment methods [UPI / card / cash].
Patient: [condition, stage, setting, what they have already asked]

Draft a reply that:
1. Answers the number question in the first two lines - no build-up.
2. Explains what the number buys: assessment, hands-on treatment, a written programme, review.
3. States the typical number of sessions as a range with the reason, and says plainly that the plan is
reviewed after [N] sessions.
4. Offers two ways to start: single session, or a package - and one sentence on why the package exists
(continuity, not pressure).
5. Ends with a single, easy next step.

Rules:
- Under 900 characters for WhatsApp.
- No discount language, no urgency, no "limited slots".
- Never promise insurance reimbursement. If they ask, use the line from prompt 40.
- Give a Hinglish variant.
```

PROMPT 32

Three-tier package proposal

Use when: When the patient is comparing you with two other clinics and needs to see the difference, not just the price.

```
You are designing a three-tier physiotherapy package for an Indian practice.

Condition and stage: [BRIEF]
My costs and time: session fee [AMOUNT], home visit [AMOUNT], my realistic capacity [patients/day]
What I actually offer that others do not: [LIST HONESTLY]
```

```
Produce three tiers with a clear name, what is included, number of sessions, duration, price, and who each tier is for:
- Tier 1: the minimum honest course (not a teaser).
- Tier 2: the standard course, the one most patients should take.
- Tier 3: the intensive or home-visit option.

Rules:
- Every tier must include: assessment, written programme, WhatsApp support between sessions, and a review at session [N].
- No tier may be designed to look cheap and then fail. If a tier cannot work clinically, say so and remove it.
- Add a one-line justification for the price of each tier that a family can repeat to each other.
- Add a "what is not included" line for each tier.
```

PROMPT 33

Objection: 'it is too expensive' / 'I will do the exercises at home'

Use when: The two objections that account for most lost patients in Indian physiotherapy.

```
You are a calm, non-pushy practice owner in India.

Patient's exact words: "[PASTE]"
Their context: [condition, severity, occupation, who pays, sessions done so far]
My fee: [AMOUNT]

Give me:
1. Three replies, ranked from softest to firmest, each under 400 characters.
2. For each reply, what it assumes about the patient - so I pick the right one.
3. The one thing I should NOT say (and why it loses the patient).
4. A fallback: the smallest useful paid step I can offer instead of the full package.
5. A one-line honest statement of what happens clinically if they stop now - without fear-mongering.

Rules:
- Never use guilt, shame, or "you will regret it".
- Never discount on the spot. If a discount is possible, offer it as a different, smaller plan.
- Give a Hinglish version of the softest reply.
```

PROMPT 34

The patient arrives with a doctor's note: '5 sittings only'

Use when: Common in India, and mishandled it either insults the doctor or under-treats the patient.

```
A patient has come with a doctor's advice of a fixed number of physiotherapy sittings.

What the note says: [PASTE OR SUMMARISE]
My assessment: [BRIEF]
What I think they actually need: [HONEST VIEW]

Give me:
1. A 60-second spoken explanation that respects the doctor's advice and explains why the plan may need to be different.
2. A written version for WhatsApp, under 500 characters.
3. A short note I can send back to the doctor explaining what I found and what I propose - one paragraph, factual.
4. The sentence to use if the patient insists on exactly the prescribed number.

Rules:
- Never criticise the referring doctor.
- Never over-ride the prescription silently. Say what you are doing and why.
- Keep the patient's choice intact: they decide, with clear information.
```

PROMPT 35

Why continue when the pain is already gone

Use when: Session four, pain down, patient thinks the job is done. This is where recurrence is created.

```
A patient's pain has settled but the course is not complete.

Condition: [DIAGNOSIS OR WORKING EXPLANATION]
What changed: [PAIN SCORE, FUNCTION, MEASURES]
What is not yet restored: [STRENGTH, RANGE, ENDURANCE, CONFIDENCE, WORK CAPACITY]
Sessions remaining: [N]

Write:
1. A 4-sentence explanation of the difference between "pain gone" and "problem solved", in everyday
words, using one analogy that fits an Indian household (do not use gym analogies for a 65-year-old).
2. A reduced plan: the smallest remaining course that protects the result, with the sessions and what
each one is for.
3. A WhatsApp message under 400 characters that offers this without pressure.
4. A one-line relapse warning the patient will remember.

Rules:
- No fear. No "you will be back in six months".
- No invented statistics about recurrence.
```

PROMPT 36

The discharge summary a patient keeps

Use when: Hand it over on the last session, in print or on WhatsApp. It reduces come-backs and earns referrals.

```
Write a discharge summary for a physiotherapy patient in India.

Case: [DE-IDENTIFIED BRIEF]
Baseline to discharge: [MEASURES BEFORE AND AFTER]
What worked: [INTERVENTIONS]
Patient's goal and whether it was met: [THEIR WORDS]
My registration number and clinic: [PLACEHOLDERS]

Structure:
1. What you came to us with (3 lines).
2. What we found and what we did (4 lines).
3. Where you are now, with the numbers (3 lines).
4. Your maintenance plan: three things to keep doing, how often, for how long.
5. Warning signs: when to come back, in plain words.
6. A line for your doctor or employer if they ask for proof of treatment.

Rules:
- Simple English, Class 8 level, under 450 words.
- Numbers as I gave them, no additions.
- No promises about the future.
```

PROMPT 37

Brief the family without briefing them against the patient

Use when: Home visits where the son, daughter-in-law or spouse is the real decision-maker.

```
You are helping me talk to a patient's family after an assessment at home.

Patient: [age band, condition, capacity to decide, cognition]
Family present: [who, and their main worry]
The honest clinical picture: [BRIEF]
The decision that has to be made: [e.g. number of sessions, home modifications, whether to continue]

Give me:
1. A 90-second opening that includes the patient, not just the family.
2. How to name the main worry out loud, so it stops driving the conversation.
3. Three sentences that separate what the patient must do from what the family must do.
```

```
4. A way to handle the family member who answers every question for the patient.
5. A closing line that leaves the decision with the patient, respectfully.

Rules:
- Respect the household hierarchy without letting it erase the patient.
- No clinical advice beyond the plan. No cost pressure in this script.
```

PROMPT 38

The first phone call for a home-visit enquiry

Use when: Ninety seconds on the phone decide whether a home visit happens at all.

```
You are a physiotherapy practice owner taking a home-visit enquiry in India.

Caller: [relationship to patient]
What I know so far: [condition, area, urgency, who asked]

Give me a call script with these beats:
1. Open with the two facts I need (condition and area) - not a form.
2. Two qualifying questions that decide whether a home visit is the right service.
3. The fee and travel answer, stated plainly, in one line.
4. What happens next: when I can come, what to keep ready, who should be present.
5. A WhatsApp follow-up message under 400 characters confirming the visit.

Rules:
- Under 200 spoken words. Sentences short enough to say on a bad line.
- If the patient needs urgent medical attention instead, the script must say so and stop selling.
- Add a Hinglish variant of the WhatsApp follow-up.
```

PROMPT 39

The patient is already seeing another physiotherapist

Use when: Handled badly this becomes a professional fight. Handled well it becomes a referral relationship.

```
A patient is considering switching physiotherapists, or taking a second opinion.

Their situation: [what they told me]
What they are unhappy about: [their words]
What I can offer: [honest description]

Give me:
1. Three questions to ask that help them decide, not to help me win.
2. A line that respects the current therapist without endorsing or criticising them.
3. What I will and will not say about another clinician's plan - as a short rule set.
4. A short message they can send to their current therapist if they decide to switch.
5. The one thing I should refuse to do, even if asked.

Rules:
- Never criticise a colleague. Never diagnose their work.
- Never start treatment on the same day as a switch without the patient's clear understanding of the
handover.
```

PROMPT 40

The insurance answer that protects you later

Use when: Because 'insurance se ho jayega' said loosely becomes a complaint.

```
A patient is asking whether health insurance will cover their physiotherapy.

What they told me: [policy type if known, whether there was a hospitalisation, dates, whether they have a
TPA]
My service: [sessions proposed, setting, fee]

Give me:
1. A short honest answer in simple English (under 400 characters) that does not promise reimbursement.
```

```
2. The three facts they should check in their own policy wording: pre-hospitalisation window,
post-hospitalisation window, and whether day-care or domiciliary cover includes physiotherapy.
3. A line explaining that out-patient clinic visits are generally not covered without a linked
hospitalisation.
4. What I will provide if they want to claim: itemised bill, my registration number, session dates, and a
treatment note - listed plainly.
5. A line for my own file recording that I did not promise coverage.

Rules:
- Do not state what any specific insurer covers.
- Do not advise them to describe the treatment in a particular way to get a claim paid.
- Keep every claim conditional on their own policy wording.
```

PROMPT 41

Explain your registration and credentials, simply

Use when: New patients check, and from 2026 the UID is the thing they should be checking.

```
Write a short credential note for patients of a physiotherapy practice in India.

My details: [DEGREE, UNIVERSITY, YEAR, STATE COUNCIL, REGISTRATION / UID NUMBER, NCAHP STATUS, YEARS OF
PRACTICE, AREAS I TREAT]

Produce:
1. A 4-line "who is treating you" block for the website and the WhatsApp business profile.
2. A version for the waiting-room wall, 6 lines, larger type.
3. A line explaining what the registration number means and how a patient can verify it.
4. A line on what I do NOT treat, so expectations are set.

Rules:
- State only what I have given you. Do not embellish the degree or invent fellowships.
- Keep it humble in tone. No "best in the city".
- No claims about success rates.
```

PROMPT 42

The informed-consent conversation, said out loud

Use when: Before manual therapy, dry needling, taping or any technique with a risk worth naming.

```
You are helping me have an informed-consent conversation for a specific technique.

Technique: [e.g. manual therapy, dry needling, taping, electrotherapy]
Patient: [age band, condition, comorbidities, medication categories, anxiety level]
Known risks I must name: [AS I UNDERSTAND THEM - I will verify]

Give me:
1. A 45-second spoken explanation in simple English: what it is, why I am suggesting it, what it feels
like, what can go wrong.
2. The three questions I must ask before proceeding, and the answer that means I should not.
3. A one-paragraph written consent note with placeholders for name, date, technique and my signature.
4. A note that this written text is a starting draft for my own legal review, not a legal document.

Rules:
- Do not add or remove clinical risks to make the conversation easier.
- Do not state complication rates.
- Say plainly that the patient can decline and still be treated.
```

PROMPT 43

Service recovery: reply to an unhappy patient

Use when: A one-star review, a complaint on WhatsApp, or a family that feels cheated.

```
You are helping me reply to an unhappy physiotherapy patient in India.

What happened: [FACTS AS YOU KNOW THEM]
```

```
Their complaint in their words: "[PASTE]"
What I can offer: [REFUND POLICY, FREE REVIEW, APOLOGY, NOTHING]
Where this is: [Google review / WhatsApp / in person]

Give me:
1. A reply that acknowledges the specific thing they are upset about - not a generic apology.
2. A version for a public Google review (under 600 characters) and a version for a private WhatsApp
message.
3. Three things I must NOT say, and the reason for each.
4. What to change in my process so this does not repeat - one concrete change.
5. A line that closes the loop without inviting an argument.

Rules:
- No defensiveness, no clinical detail about the patient in a public reply.
- Never disclose that they were a patient in a public space without their consent.
- Do not admit liability for a clinical outcome; acknowledge the experience and offer a review.
```

PROMPT 44

Audit my own WhatsApp replies

Use when: Once a month. Paste ten of your sent messages and read what your practice sounds like.

```
Act as a communication reviewer for an Indian physiotherapy practice.

Here are ten messages I sent to patients this month (de-identified):
[PASTE]

Review them on:
1. Clarity - could a patient who is in pain understand it in one reading?
2. Tone - does it sound like a person or a template?
3. Length - which ones should have been three lines shorter?
4. The money question - was the fee ever left ambiguous?
5. The next step - did every message end with one clear next step?
6. Safety - any clinical advice that should not have gone over WhatsApp?

Then rewrite the two weakest messages. Keep my voice: [DESCRIBE HOW YOU TALK - e.g. warm, direct, no
emojis].
```

PART 7

07

Operations: the evening that disappears

Fourteen prompts that rebuild the admin half of the practice - intake, reminders, records, bills, SOPs and the data you keep.

IN THIS PART

45 The intake form that does not waste the first session
46 The appointment confirmation and reminder sequence
47 No-show recovery that does not shame the patient
48 Cancellation and refund policy, in language patients accept
49 Home-visit route and slot planning logic
50 The package balance message
51 The itemised bill note for a patient who wants to claim
52 Consent form draft for treatment and for data

and 6 more in this part

Software does the scheduling. What software does not do is write the wording, design the form, draft the SOP or decide what to keep. That is the hour these prompts give back.

PROMPT 45

The intake form that does not waste the first session

Use when: Design it once, use it for years.

```
Design a patient intake form for an Indian physiotherapy practice.

My setting: [clinic / home visit / both]
What I must know before the first session: [LIST]
What I always forget to ask: [LIST]

Produce a form with these sections, in this order:
1. Identity and contact (minimum necessary - no Aadhaar, no full address for clinic patients)
2. Consent to treatment and consent to WhatsApp communication (separate tick boxes)
3. Reason for visit in the patient's own words
4. History: onset, previous treatment, medication categories, surgeries, comorbidities
5. Function: work, sleep, walking, self-care, exercise currently done
6. What the patient wants to be able to do again
7. How they heard about us

Rules:
- One page, printable, with fields a patient can fill in 4 minutes.
- No field that I will never read.
- Add a note under the data section: what we collect, why, and how long we keep it - three lines of plain language.
- Add a Hinglish version of the section headings.
```

PROMPT 46

The appointment confirmation and reminder sequence

Use when: Written once, used every day, and it cuts no-shows more than any app.

```
You are writing WhatsApp messages for a physiotherapy practice in India.

Practice details: [clinic name, area, timings, home-visit availability]
Appointment: [day, time, type]

Write four messages:
1. Confirmation immediately after booking (under 300 characters) - include day, date, time, place, and what to bring.
2. Reminder the evening before (under 250 characters).
3. Reminder 2 hours before, for home visits (under 200 characters) - include travel and a "call if the gate is locked" line.
4. Post-visit follow-up the next morning (under 300 characters) - how it felt, and the next appointment.

Rules:
- Date in Indian format: DD/MM/YYYY, day named.
- No emojis. No marketing in a reminder.
- Every message ends with one action: reply YES, reply 1, or call.
- Add a Hinglish variant of message 2.
```

PROMPT 47

No-show recovery that does not shame the patient

Use when: Two messages, spaced, and a rule for the third.

```
A patient did not attend a physiotherapy appointment.

Context: [first visit or repeat, how many no-shows, their condition, whether they paid in advance]
Practice policy: [what I want to enforce - e.g. no charge for the first miss]

Write:
```

```
1. A message sent 30 minutes after the missed slot (under 250 characters) - no blame, one question.
2. A message sent the next day (under 250 characters) - offering two specific slots.
3. A message sent after a second no-show (under 300 characters) - stating the policy calmly.
4. My internal rule: after how many no-shows I stop holding the slot, and the exact wording.
5. A version for a patient who has paid for a package, where the session is deducted.

Rules:
- Never imply the patient does not care about their health.
- Never use "as per policy" as the whole message.
- Give a Hinglish variant of message 1.
```

PROMPT 48

Cancellation and refund policy, in language patients accept

Use when: The policy nobody reads until there is a dispute.

```
Draft a cancellation and refund policy for a physiotherapy practice in India.

My position on: [cancellation notice, home-visit travel charges, package expiry, refunds for unused
sessions, refunds for digital programmes]

Produce:
1. A plain-language policy of 8 bullet points, each under 20 words.
2. A one-paragraph version for the website.
3. A three-line version for the bottom of a WhatsApp confirmation.
4. The two situations where I should waive the policy, and the wording for each.
5. A note on what my policy must not say, so it does not conflict with consumer protection norms - flag
it for my own legal review.

Rules:
- No clause that punishes a patient for illness or a family emergency.
- No "no refunds under any circumstances".
- Clear, not legalistic.
```

PROMPT 49

Home-visit route and slot planning logic

Use when: Because a 45-minute session becomes two hours in an Indian city, and the diary never knows it.

```
You are an operations planner for a home-visit physiotherapy practice.

My city and areas: [LIST AREAS YOU COVER]
Typical travel time between areas: [HONEST ESTIMATES]
Session length: [MINUTES]
My working window: [HOURS]
Patients needing daily visits: [N]

Produce:
1. A weekly slot template that clusters visits by area, with travel blocks shown.
2. The rule for how many home visits a day is realistic, given travel and note-writing.
3. What to do with the patient who insists on a time that breaks the cluster.
4. A pricing adjustment rule for the far-zone patient, stated as a zone fee rather than a surcharge.
5. A one-line message explaining travel-time billing to a new patient, without sounding like an excuse.

Rules:
- Assume real traffic, not map estimates.
- Leave 15 minutes of buffer in the day, named in the template.
- No promises about arrival times to the minute.
```

PROMPT 50

The package balance message

Use when: Sessions are money and money is trust. Send this before the patient has to ask.

```
You are writing a package balance update for a physiotherapy patient in India.

Package: [N sessions for AMOUNT]
Sessions used: [N]
Sessions remaining: [N]
Patient's progress in one line: [PLAIN WORDS]
Next appointment: [DATE]

Write:
1. A WhatsApp message under 350 characters: sessions used, remaining, progress line, next appointment.
2. A one-line version for a printed bill.
3. A message for the patient whose package is expiring in [N] days, with two honest options.
4. My internal ledger format: a 6-column table I can keep in a notebook or a sheet.

Rules:
- State the remaining number in the first line.
- Never invent a balance. If I have not given it, ask me.
- No upselling in the same message. Give the balance, then stop.
```

PROMPT 51

The itemised bill note for a patient who wants to claim

Use when: One page, correct, and it saves a twenty-minute argument at the counter.

```
Draft an itemised physiotherapy bill note for a patient in India.

My details: [CLINIC NAME, ADDRESS, PHONE, COUNCIL, REGISTRATION / UID]
Patient: [NAME PLACEHOLDER], [DATES OF SESSIONS], [SESSIONS AND TYPE]
Charges: [ITEMISED WITH AMOUNTS]
Tax position: [WHETHER GST APPLIES AND AT WHAT RATE - I WILL CONFIRM MY OWN POSITION]

Produce:
1. A bill layout with columns: Date | Description | Sessions | Rate | Amount.
2. A treatment summary block: condition, period of treatment, number of sessions, outcome measures with values.
3. A line stating what this document is and is not (it is a treatment and payment record, not a guarantee of insurance reimbursement).
4. A signature block with my registration number and the note that it can be verified with the State Council.

Rules:
- Do not calculate or invent any tax rate. Mark [MY TAX POSITION] and leave it to me.
- Do not add a diagnosis I did not supply.
- Keep it to one page.
```

PROMPT 52

Consent form draft for treatment and for data

Use when: A working draft for you to take to your own lawyer - not a legal document.

```
Draft a consent form for a physiotherapy practice in India.

What I need consent for: [assessment and treatment / a specific technique / sharing records with a referring doctor / using WhatsApp for clinical communication / photographing for progress records]
Patient category: [adult / minor with guardian / elderly with cognitive concerns]

Produce:
1. A one-page form in plain English with separate, unticked consent boxes - never one blanket box.
2. A short explanation above each box: what I am asking, why, and what happens if they say no.
3. A section on data: what is collected, who sees it, where it is stored, how long it is kept, how to ask for deletion.
```

```
4. A withdrawal-of-consent line.
5. A clear note at the top: this is a draft for review by my own legal advisor.

Rules:
- No consent by silence or by default.
- No clause that takes away the patient's right to complain.
- Keep the reading level simple.
```

PROMPT 53

The clinical note template that takes 90 seconds

Use when: SOAP in a shape that suits a physiotherapy session, not a physician's ward round.

```
Design a physiotherapy session note template for an Indian practice.

My setting: [clinic / home visit]
What I must record every session: [LIST, INCLUDING WHAT A COUNCIL OR COURT MIGHT ASK FOR]
What I currently forget: [LIST]

Produce:
1. A one-screen template: Date | Session no. | Subjective (2 lines) | Objective (measures with values) |
Assessment (my reasoning, one line) | Plan (what changes) | Advice given | Next review.
2. Shorthand prompts beside each field so I can type fast on a phone.
3. The five fields that must never be blank, and why.
4. A 20-second end-of-day routine that closes any gaps in yesterday's notes.

Rules:
- Keep it to one screen. If it does not fit on a phone, I will not use it.
- No field that duplicates the intake form.
- Include a reminder line: "record facts, not opinions about the patient".
```

PROMPT 54

The weekly schedule audit

Use when: Fifteen minutes on a Sunday, and you find the leak before the month ends.

```
You are my practice operations analyst.

Last week's data: [SLOTS OFFERED, SLOTS FILLED, NO-SHOWS, CANCELLATIONS, NEW ENQUIRIES, CONVERSIONS,
HOME VISITS, TRAVEL HOURS, REVENUE]

Analyse:
1. Fill rate by day and by time block - where the diary actually leaks.
2. The cost of no-shows in rupees this week.
3. The two changes with the highest likely impact next week, in order.
4. One question I should ask every patient who did not convert.
5. A single metric to track for the next four weeks.

Rules:
- Use only my numbers. Do not estimate or fill gaps with industry averages.
- If the data is too thin to conclude, say so plainly and tell me the two numbers to start recording.
```

PROMPT 55

Consumables and equipment reorder list

Use when: For the clinic that runs out of tape on the busiest Saturday of the month.

```
You are helping me set up a simple stock routine for a physiotherapy practice in India.

What I use: [LIST: tape, gel, needles, bands, electrodes, towels, sanitizer, printing, etc.]
Monthly patient volume: [N]
Storage space: [HONEST DESCRIPTION]
What runs out unexpectedly: [LIST]

Produce:
```

```
1. A reorder table: Item | Unit | Typical monthly use | Reorder when stock falls to | Supplier type (not brand).
2. A five-minute monthly stock check routine.
3. Which items I should never buy in bulk, and why.
4. A simple way to record usage per session so the numbers are real.

Rules:
- No brand names and no prices - I will price locally.
- Assume small-scale storage: a cupboard, not a store room.
```

PROMPT 56

The home-visit safety SOP

Use when: Written once, followed every visit. The one SOP that protects you personally.

```
Write a safety SOP for home physiotherapy visits in India.

My context: [city, areas, whether I visit alone, hours, patient profile]
What has worried me before: [LIST]

Produce an SOP with these sections, each as short numbered steps:
1. Before leaving: what I confirm on the phone, what I share with my family or clinic, and the check-in time.
2. On arrival: how I assess the home for safety before starting.
3. During the visit: three rules that do not bend (e.g. no lifting a patient alone).
4. If something goes wrong: the escalation sequence, with who I call first.
5. After the visit: the 60-second log I write before driving off.
6. The list of situations where I decline or stop a home visit.

Rules:
- Practical, not corporate. Written for one person doing the work.
- Include a one-page version I can keep in the car.
- No legal claims - flag anything I should check with my own advisor.
```

PROMPT 57

New patient onboarding SOP

Use when: So the first fifteen minutes are the same every time, and no step depends on your memory.

```
Design a new patient onboarding SOP for a physiotherapy practice in India.

My process today: [DESCRIBE WHAT ACTUALLY HAPPENS, HONESTLY]
Where it breaks: [LIST]

Produce:
1. A step list from first message to end of first session, with who does what and how long it takes.
2. The four documents the patient receives, and when.
3. The two points where patients usually drop off, and the message that saves each.
4. A checklist the front desk or I can tick in 30 seconds.
5. The first-session script opening and closing, in plain English.

Rules:
- Fit it inside a 45-minute session. Do not design a 90-minute ideal.
- No step that needs software I may not have.
- Include a Hinglish version of the closing lines.
```

PROMPT 58

Data retention and deletion routine

Use when: The unglamorous half of data protection, written as a routine you can actually keep.

```
You are helping me set up a simple patient-data routine for a solo physiotherapy practice in India.

What I hold: [WHATSAPP CHATS, PAPER FILES, PHOTOS, BILLS, REFERRAL LETTERS, APPOINTMENT RECORDS]
Where: [PHONE, LAPTOP, CLOUD, CUPBOARD]
```

```
How long I keep it today: [HONEST ANSWER]

Produce:
1. A retention table: Record type | Where kept | Who can access | Keep for | Then what.
2. A monthly 10-minute routine: what to delete, what to archive, what to check.
3. The plain-language line I give patients about their data, for the intake form and the website.
4. A one-page note on what to do if a patient asks for a copy of their records, or for deletion, and
where the limits of deletion sit for clinical records.
5. A short list of things I should stop doing today (e.g. patient photos in a personal gallery).

Rules:
- Practical, not aspirational. If a step needs a tool I do not have, offer the paper alternative.
- Flag anything that needs a lawyer's or accountant's view rather than answering it as settled law.
```

PART 8

08

Growth: getting found, getting referred

Fourteen prompts for the local search, the review, the doctor's referral and the apartment society that actually bring patients.

IN THIS PART

59 Google Business Profile that ranks for your area
60 Ask for a review without making it awkward
61 Reply to every kind of review
62 The area landing page that brings local patients
63 A 30-day content calendar you will actually keep
64 Three reel scripts that do not need a studio
65 Before/after posts without breaking trust
66 The letter that gets you doctor referrals

and 6 more in this part

Physiotherapy is a local, trust-led purchase. Almost every prompt in this part is about being findable and being credible in your own two-kilometre radius - not about running ads.

PROMPT 59

Google Business Profile that ranks for your area

Use when: The single highest-return free asset a physiotherapy practice has.

```
You are a local SEO specialist for healthcare practices in India.

My practice: [CLINIC NAME], [AREA, CITY], [SERVICES], [HOME VISITS YES/NO], [YEARS], [TIMINGS],
[LANGUAGES SPOKEN]
What I want to be found for: [e.g. "physiotherapist near Kothrud", "home physiotherapy Pune"]

Produce:
1. A business description under 750 characters, written for a patient, not for a search engine.
2. The primary category and up to five secondary categories to select, with the reason for each.
3. Ten services to list, each with a one-line description a patient understands.
4. Five posts for the next five weeks, each under 300 characters.
5. The five attributes to switch on (e.g. wheelchair accessible, home visits, online appointments).
6. Three things that hurt local ranking, and the fix for each.

Rules:
- No keyword stuffing. No "best in city" claims.
- No promises of ranking positions.
- Assume a solo practitioner with no marketing budget.
```

PROMPT 60

Ask for a review without making it awkward

Use when: At the right moment, with the right words, most happy patients will say yes.

```
Write a review request for a physiotherapy patient in India.

Context: [session number, outcome in plain words, relationship with the patient, whether they are on
WhatsApp]
My Google Business Profile link: [PLACEHOLDER]

Produce:
1. The best moment to ask, in the treatment course, and why.
2. A WhatsApp message under 300 characters that asks directly.
3. A spoken version, 20 seconds, for the end of the session.
4. A message for the patient who says yes but does not do it - one nudge, then stop.
5. A Hinglish variant.

Rules:
- No incentive, no discount, no gift in exchange for a review.
- Never write the review for them or suggest what to say.
- Never ask a patient who is still mid-course and in pain.
```

PROMPT 61

Reply to every kind of review

Use when: Five replies you can adapt, so you never freeze in front of a one-star.

```
You are writing review replies for a physiotherapy practice in India.

My practice: [NAME, AREA, TONE YOU WANT - e.g. warm and direct]
Write replies for these five situations:
1. Five stars, no text.
2. Five stars, detailed, mentions my staff by name.
3. Three stars, says "treatment was okay but expensive".
4. One star, says "no improvement after 10 sessions".
5. A review that appears to be from someone who was never a patient.
```

```
For each: a reply under 500 characters, plus a one-line note on what NOT to do.

Rules:
- Never disclose clinical details about the patient in a public reply.
- Never argue about the clinical outcome in public - move it to a private channel.
- Never admit liability. Acknowledge the experience, offer a review.
- For number 5, include how to request removal without naming the person.
```

PROMPT 62

The area landing page that brings local patients

Use when: One page per locality you serve. Written once, ranks for years.

```
Write a local landing page for a physiotherapy practice in India.

Locality: [AREA, CITY]
What I offer there: [SERVICES, HOME VISITS, TIMINGS]
Who lives there: [e.g. young families in towers, retired residents in bungalows, students in hostels]
Travel time from my clinic: [MINUTES]

Produce:
1. A page title under 60 characters and a meta description under 155.
2. An H1 and four H2 sections: what I treat here, how home visits work in this area, what a first session looks like, how to book.
3. Two paragraphs of genuinely local detail - landmarks and traffic realities, not filler.
4. A FAQ block of five questions people in this area actually ask.
5. An internal-link plan: which of my other pages this one should link to.

Rules:
- No invented patient numbers, no "500+ patients treated" unless I supply it.
- No duplicate text across localities - each page must be written for that area.
- Plain English, short sentences.
```

PROMPT 63

A 30-day content calendar you will actually keep

Use when: For the physiotherapist who knows content matters but has no plan on Sunday night.

```
Build a 30-day social media calendar for a physiotherapy practice in India.

Platforms: [Instagram / YouTube Shorts / Facebook / LinkedIn]
My strengths: [e.g. home visits, elderly rehab, sports, post-surgical]
My comfort on camera: [none / voice only / face on camera]
Time available: [MINUTES PER WEEK]

Produce:
1. A table: Day | Format | Topic | Hook line | What to show | Caption opening line.
2. Four repeatable formats I can film in one sitting and post for a month.
3. The three topics that get the most saves for physiotherapists, and why.
4. What I should never post (patient privacy, before/after rules, claims).
5. A batching routine: what to shoot, in what order, in 90 minutes.

Rules:
- Nothing that requires editing skills beyond trimming a clip.
- No dancing, no trends that do not fit a clinic.
- Every topic must be something I can answer from my own practice.
```

PROMPT 64

Three reel scripts that do not need a studio

Use when: Filmed on a phone, in the clinic, between patients.

```
Write three short video scripts for a physiotherapy practice in India.

Topic area: [e.g. knee pain, back pain at desk, post-surgery walking]
```

```
My setting: [clinic room / patient's home]
Length: [30 to 45 seconds]

For each script give:
1. The hook, first 3 seconds, spoken word for word.
2. Shot list: 4 to 6 shots, each one line, filmable with a phone on a stand.
3. The voiceover or on-camera lines, in simple English.
4. The on-screen text for each shot (maximum 6 words each).
5. The caption and three hashtags that are relevant rather than popular.
6. A Hinglish version of the hook.

Rules:
- No patient may appear without written consent - write the scripts so a colleague or a family member demonstrates.
- No medical claims, no "cure", no before/after promises.
- Every script must end with one clear action.
```

PROMPT 65

Before/after posts without breaking trust

Use when: Progress content converts well and is the easiest way to lose your registration and your reputation.

```
You are a healthcare communications advisor in India.

I want to post progress content. My material: [e.g. range of motion improvement, walking distance, return to gym]
Patient consent status: [written consent obtained / not yet / anonymous]

Produce:
1. The consent checklist I need before any patient image or story goes up - five items.
2. Three ethical ways to show progress without showing a patient's face.
3. A post template that shows a measured change with the number, the period and the caveat.
4. The five things I must never post.
5. A line for my consent form that covers social media specifically.

Rules:
- No "miracle" framing. No claims that the result is typical.
- No images of a patient who cannot fully consent (children without guardian consent, cognitively impaired adults).
- No identifying details even in text.
```

PROMPT 66

The letter that gets you doctor referrals

Use when: One page, hand-delivered, followed by a visit. Still the strongest channel in Indian physiotherapy.

```
Write a referral-introduction letter from a physiotherapy practice to a doctor in India.

Me: [NAME, QUALIFICATION, REGISTRATION, YEARS, AREAS OF FOCUS, SETTING, HOME VISITS YES/NO]
The doctor: [SPECIALITY, HOSPITAL OR CLINIC, AREA]
Why I am writing to them specifically: [HONEST REASON]
What I can offer their patients: [SPECIFIC, NOT "BEST CARE"]

Produce:
1. A one-page letter under 300 words, formal but warm.
2. A half-page "what I will send back to you" note - the referral reply they can expect.
3. Three talking points for the in-person visit that follows.
4. A follow-up message for two weeks later, under 150 words.
5. What I must never say about a competing physiotherapist in that conversation.

Rules:
- No flattery, no exaggeration of qualifications.
- No request for "all your patients". Ask for the specific case types I am good at.
- Include my registration number - in 2026 that is the credential a doctor will check.
```

PROMPT 67

The apartment society camp proposal

Use when: Two hundred families behind one gate, and a committee that decides in one meeting.

```
Write a proposal for a physiotherapy camp or talk in an apartment society in India.

Society: [NAME, AREA, APPROXIMATE FLATS, RESIDENT PROFILE]
What I propose: [e.g. free posture screening, balance assessment for seniors, a 45-minute talk]
What I need from them: [space, a table, permission to display, a WhatsApp broadcast]
What I get: [honest description]

Produce:
1. A one-page proposal with a clear title, the benefit to residents, the logistics, and what I need.
2. A WhatsApp message to the society secretary under 400 characters.
3. The 45-minute session outline for a seniors' group, with three practical demonstrations.
4. A follow-up message to residents who attended, under 300 characters.
5. The three things that go wrong at these camps, and how I avoid each.

Rules:
- No hard selling on the day. Screenings are screenings, not sales.
- No free treatment promises that I cannot repeat for everyone.
- Include a data note: what I record, and that residents can decline.
```

PROMPT 68

The corporate ergonomics pitch

Use when: Offices pay for this, and one workshop can produce a year of individual patients.

```
Write a corporate workshop pitch for a physiotherapy practice in India.

Target company: [SIZE, SECTOR, LOCATION]
My offer: [e.g. 60-minute desk ergonomics session for 30 employees, plus individual screening slots]
My price: [AMOUNT OR RANGE]

Produce:
1. A one-page proposal: the problem (with no invented statistics), the session, the outcomes they can
measure, the price, the logistics.
2. An email to HR under 200 words.
3. A follow-up message for one week later.
4. The session outline with timings.
5. A one-page handout to leave behind, with five desk fixes.
6. What I must not claim about productivity or sick days.

Rules:
- No fabricated research, no "studies show 40 per cent".
- Price stated once, plainly, without apology.
- Offer a small paid pilot rather than a free full session.
```

PROMPT 69

The reactivation campaign for past patients

Use when: Your cheapest patient is the one who already trusted you.

```
Write a reactivation campaign for a physiotherapy practice in India.

Past patients: [NUMBER, TYPICAL CONDITIONS, HOW LONG SINCE LAST VISIT]
Season or trigger: [e.g. winter joint stiffness, monsoon slips, year-end desk marathon]
What I am offering: [e.g. a review session, a re-assessment, a seasonal programme]

Produce:
1. A three-message sequence over ten days, each under 350 characters, each with a different angle.
2. A Hinglish variant of the first message.
3. The list segmentation: who I message, who I do not.
4. A reply-handling guide for the four likely answers (yes, later, no, "who is this").
5. A rule for how often I can run this without becoming spam.
```

```
Rules:
- No fear language about their old injury.
- No blanket discount. Offer a review, not a clearance sale.
- Include an opt-out line.
```

PROMPT 70

A seasonal offer that is clinically honest

Use when: Winter is joint-pain season in most of India. That is a real reason, not a marketing trick.

```
Design a seasonal offer for a physiotherapy practice in India.

Season: [winter / monsoon / summer]
What genuinely changes for my patients: [HONEST CLINICAL OBSERVATION]
My capacity: [PATIENTS PER DAY, HOME VISITS POSSIBLE]
My fee structure: [AMOUNTS]

Produce:
1. The offer, in one sentence a patient can repeat.
2. What is included and what is not, in two short lists.
3. The price, with the reasoning stated in one line.
4. A WhatsApp message and a Google Business post.
5. The capacity limit - when I stop taking this offer, stated publicly.
6. The three ways this offer could go wrong operationally, and the fix.

Rules:
- The offer must be something I can deliver at my current capacity.
- No countdowns, no fake scarcity. A real capacity limit is fine; an invented one is not.
- No claim that the offer prevents or cures anything.
```

PROMPT 71

The website FAQ block that answers the real questions

Use when: Also the block that gets you into the answer box on a search.

```
Write an FAQ block for a physiotherapy practice website in India.

My practice: [SETTING, SERVICES, FEES, AREAS, TIMINGS]
Questions patients actually ask me: [LIST TEN, IN THEIR WORDS]

Produce:
1. Twelve questions in the patient's own phrasing, with answers under 80 words each.
2. Each answer structured as: direct answer in the first line, then the detail.
3. A short note under the block: what to do if the answer is not here.
4. Markup-ready structure: question heading, answer paragraph, in order.

Rules:
- No question that exists only to stuff a keyword.
- No answer that promises an outcome or an insurance reimbursement.
- Every fee answer must match my stated fees exactly.
```

PROMPT 72

The testimonial request with consent built in

Use when: Useful on the website, safe for you, respectful to the patient.

```
Write a testimonial request and consent note for a physiotherapy practice in India.

Patient context: [condition category, outcome in plain words, relationship]
Where the testimonial will appear: [website / Google / social media / print]

Produce:
1. A WhatsApp request under 300 characters.
```

```
2. A consent note of four lines that the patient replies to with a "yes", covering: what will be
published, where, for how long, and how to withdraw.
3. A prompt to help the patient write it in their own words - three questions, not a script.
4. What I must do if the testimonial includes a photo.
5. A Hinglish variant of the request.

Rules:
- Never write the testimonial for them.
- No incentive of any kind.
- No clinical claim in the published version that I cannot support.
```

PART 9

09

Team and money

Eight prompts for the year you stop being a one-person practice - hiring, training, pricing and the numbers.

IN THIS PART

73 The job description an associate physio will answer
74 Interview questions that predict how they treat patients
75 The first 30 days for a new hire
76 Train the assistant who is not a physiotherapist
77 Price your sessions with a number you can defend
78 The monthly numbers review
79 Break-even before you open a second location
80 The capacity plan for the year you stop doing everything

PROMPT 73

The job description an associate physio will answer

Use when: Most physiotherapy job posts are unreadable. This one is not.

```
Write a job description for a physiotherapy position in India.

Role: [associate physiotherapist / home-visit physiotherapist / clinic assistant]
My practice: [SETTING, AREAS, PATIENT PROFILE, TEAM SIZE]
Salary and structure: [FIXED / FIXED PLUS SHARE / PER SESSION], [RANGE]
What the person will actually do daily: [HONEST LIST]

Produce:
1. A posting under 400 words: the practice in 3 lines, the day in 5 bullets, what I am looking for in 4 bullets, what I offer in 4 bullets, how to apply.
2. A short version for a WhatsApp group or a college noticeboard, under 200 words.
3. The three requirements that are genuinely non-negotiable (including registration), and three that are negotiable.
4. The one line that filters out people who will not last.

Rules:
- No "passionate self-starter" language. Say what the work is.
- State the salary structure plainly. Hidden pay wastes everyone's time.
- No requirement that is not actually required for the job.
```

PROMPT 74

Interview questions that predict how they treat patients

Use when: Eight questions, a scorecard, and a rule for the reference check.

```
You are helping me interview for a physiotherapy role in India.

Role: [DESCRIBE]
What matters most in my practice: [e.g. home visits, elderly care, patient communication, documentation]
My past hiring mistakes: [HONEST LIST]

Produce:
1. Eight interview questions, each with what a good answer contains and what a weak one sounds like.
2. A practical task of 15 minutes I can set in the clinic (e.g. explain a programme to a role-playing patient).
3. A scoring sheet: five criteria, 1 to 5 each, with the definition of a 5.
4. The two reference-check questions that reveal the most.
5. The red flags that should end the interview, respectfully.

Rules:
- No questions about marriage, family plans, caste, religion or community.
- Every question must relate to the work.
- Include one question that tests how they handle a patient who cannot pay.
```

PROMPT 75

The first 30 days for a new hire

Use when: Onboarding is where most small practices lose a good physiotherapist in month two.

```
Design a 30-day onboarding plan for a physiotherapy practice in India.

Role: [DESCRIBE]
What they must be able to do alone by day 30: [LIST]
What they shadow first: [LIST]
My time available to teach: [HOURS PER WEEK]

Produce:
1. A week-by-week plan: Week 1 shadowing and basics, Week 2 supervised practice, Week 3 independent with review, Week 4 full load with feedback.
2. A checklist of competencies, each observable.
3. The five things I must tell them in the first hour.
```

```
4. A 15-minute weekly check-in structure with three questions.
5. The conversation script for day 20, where problems usually surface.

Rules:
- Realistic for a busy clinic: no more than 2 hours of formal teaching a week.
- Include documentation and consent as day-one competencies, not afterthoughts.
- Include how they use AI tools responsibly in my practice - three rules.
```

PROMPT 76

Train the assistant who is not a physiotherapist

Use when: The person who actually decides whether your patients feel looked after.

```
Write a training plan for a clinic assistant or physiotherapy attendant in India.

Their tasks: [e.g. greeting, appointments, billing, machine set-up, chaperoning, home-visit driving]
What they must never do: [LIST]
Their language and literacy: [HONEST DESCRIPTION]
Time available for training: [HOURS]

Produce:
1. A one-page task sheet in simple English, plus a Hinglish version.
2. A three-session training plan with what to demonstrate and what to practise.
3. The five phrases that make a patient feel respected, and the five that do not.
4. A script for the two hardest moments: asking for payment, and handling a walk-in when the diary is
full.
5. A weekly 10-minute review routine.

Rules:
- Assume no clinical background. No medical terms without a plain gloss.
- Every task must have a clear boundary: what to do, and when to call me.
- Nothing about clinical decision-making.
```

PROMPT 77

Price your sessions with a number you can defend

Use when: Not a formula from the internet - a calculation using your own costs and hours.

```
You are a pricing analyst for a small physiotherapy practice in India.

My numbers: fixed monthly costs [RENT, SALARY, EMI, SOFTWARE, ELECTRICITY], variable cost per session
[CONSUMABLES, TRAVEL], sessions I can realistically deliver per week [N], weeks I work per year [N], my
target monthly income [AMOUNT], current fees [AMOUNTS], what local clinics charge [RANGE YOU KNOW].

Produce:
1. My true cost per session, shown as a calculation with the arithmetic visible.
2. The minimum fee that covers costs at 70 per cent utilisation.
3. Three fee structures: single session, package, home visit - with the reasoning for each.
4. What happens to the numbers if I raise fees 15 per cent and lose 10 per cent of patients.
5. The one number I must track monthly to know if this is working.

Rules:
- Show the arithmetic. Do not give me an industry benchmark.
- Do not recommend a price - give me the options and the trade-offs.
- Flag any input I have left blank rather than assuming it.
```

PROMPT 78

The monthly numbers review

Use when: Twenty minutes, once a month, with the four numbers that matter for a practice this size.

```
You are my practice finance reviewer.

This month: revenue [AMOUNT], sessions delivered [N], new patients [N], returning patients [N], no-shows [N], home visits [N], total expenses [AMOUNT], outstanding payments [AMOUNT], biggest expense [WHAT AND HOW MUCH].

Last month, the same figures: [PASTE]

Produce:
1. The four numbers that changed most, and what each change probably means.
2. Revenue per session and per patient, this month against last.
3. The single biggest financial leak visible in these numbers.
4. One change to make next month, and the metric that will show whether it worked.
5. The two numbers I am not tracking that I should start tracking.

Rules:
- Use only my numbers. Say "not enough data" where it is true.
- No investment, tax or legal advice - flag those questions for my CA.
- Keep it under one page.
```

PROMPT 79

Break-even before you open a second location

Use when: Or before you hire a second physiotherapist. The same arithmetic, either way.

```
You are evaluating an expansion decision for a physiotherapy practice in India.

Current practice: [REVENUE, PATIENTS, CAPACITY, FIXED COSTS]
The expansion: [second clinic / home-visit team / hiring an associate]
Its costs: [RENT, SALARY, EQUIPMENT, DEPOSIT, SET-UP, MARKETING]
Expected volume in months 1 to 6: [HONEST ESTIMATE]
My cash buffer: [AMOUNT]

Produce:
1. Monthly break-even in sessions and in patients, with the arithmetic shown.
2. A six-month cash requirement table, worst case and expected case.
3. The three assumptions that would sink this, and how I would test each cheaply first.
4. A smaller version of this expansion that tests the same demand for a tenth of the cost.
5. The single number that tells me to stop, and when to check it.

Rules:
- Show all arithmetic. No optimism bias.
- Do not tell me whether to expand. Lay out the trade-off.
- Flag any blank input rather than filling it with an average.
```

PROMPT 80

The capacity plan for the year you stop doing everything

Use when: Which tasks leave your hands first, and what it costs to hand them over.

```
You are a practice operations planner.

What I do in a week: [LIST ALL TASKS, INCLUDING THE SMALL ONES]
Hours per week on each: [HONEST ESTIMATES]
My revenue per clinical hour: [AMOUNT]
What I want more of: [e.g. clinical hours, home visits, teaching, family time]

Produce:
1. A table splitting my week into clinical, admin, marketing, and other - with hours.
2. The three tasks to hand over first, ranked by hours saved against the cost of handing them over.
3. What each handover needs: a written SOP, a tool, or a person.
```

```
4. A 90-day sequence: what I stop doing in month 1, 2 and 3.
5. The one task I should never hand over, and why.

Rules:
- Assume I cannot hire three people. Plan one change at a time.
- Price handovers in rupees using my own numbers.
- No productivity platitudes.
```

PART 10

10

Seven WhatsApp scripts, ready to send

The messages you send most often, already written in the tone Indian patients reply to. Replace the brackets, send.

IN THIS PART

- S1 New enquiry, first reply
- S2 Appointment confirmation
- S3 The day-before reminder
- S4 After the first session
- S5 Package balance update
- S6 Discharge message
- S7 Reactivation, sent once a season

These are scripts, not prompts: the wording is finished. Keep them as saved replies in WhatsApp Business so you send them in two taps. Change the numbers, keep the structure.

SCRIPT S1

New enquiry, first reply

Use when: Within five minutes of any enquiry. Speed matters more than polish.

```
Namaste [NAME], this is [YOUR NAME], physiotherapist at [CLINIC], [AREA].

To guide you properly I need two things:
1. What is the problem, and since when?
2. Is the patient able to travel to the clinic, or do you need a home visit?

Consultation is [AMOUNT] and takes about [MINUTES] minutes. After that I will tell you honestly how many sessions are usually needed.

You can also send a voice note if that is easier.
```

SCRIPT S2

Appointment confirmation

Use when: Immediately after the slot is agreed.

```
Confirmed: [DAY], [DD/MM/YYYY] at [TIME] at [CLINIC / HOME VISIT].

Please bring:
- Any reports or scans related to this problem
- A list of medicines currently being taken
- Loose clothing that allows movement of the [AREA] area

If you are running late, message me here. If you need to change the slot, [N] hours' notice helps me offer it to someone else.

See you on [DAY].
```

SCRIPT S3

The day-before reminder

Use when: The evening before, one message, no marketing.

```
Reminder: your physiotherapy session is tomorrow, [DAY] [DD/MM] at [TIME], [CLINIC / your home].

Reply YES to confirm, or call [NUMBER] if you need to change it.

[For home visits only: please keep the parking or gate access clear, and have one family member present if possible.]
```

SCRIPT S4

After the first session

Use when: The next morning. This single message does more for retention than any discount.

```
Good morning [NAME]. How is the [AREA] feeling today compared to yesterday?

Reply:
1 - Better
2 - Same
3 - More pain

If it is 2 or 3, I will adjust the programme before your next session. Either way, please do only the exercises marked for week 1 - not more.
```

SCRIPT S5

Package balance update

Use when: Send it before the patient asks. Once a month, or when three sessions remain.

```
[NAME], a quick update on your package:

Sessions used: [N] of [TOTAL]
Remaining: [N]

Progress so far: [ONE HONEST LINE, e.g. "walking distance is up from 200 m to 600 m"].

Next session: [DAY] [DD/MM] at [TIME]. After that we will review whether you need the full remaining set or fewer.
```

SCRIPT S6

Discharge message

Use when: On the last session, along with the printed summary.

```
[NAME], today was your last planned session. Your discharge summary is attached.

Keep doing these three things:
1. [EXERCISE] - [DOSAGE]
2. [ACTIVITY] - [HOW OFTEN]
3. [HABIT] - [WHEN]

Come back if: [WARNING SIGN 1], [WARNING SIGN 2], or if the [AREA] pain returns and lasts more than a week.

You can message me here if something comes up. I am glad the [GOAL] is working out.
```

SCRIPT S7

Reactivation, sent once a season

Use when: To past patients who completed a course. Once, not repeatedly.

```
[NAME], this is [YOUR NAME] from [CLINIC]. It has been about [N] months since your [CONDITION] treatment.

No sales pitch - just two things:
1. How is the [AREA] holding up?
2. If you have stopped the maintenance exercises, this is a good week to restart them.

If you would like a 20-minute review to check your form, reply REVIEW and I will send two slots. If you would rather not receive these messages, reply STOP and I will remove you.
```

PART 11

11

The routines

Three routines - daily, weekly, monthly - that turn eighty prompts into a habit instead of a bookmarked page.

11.1 The 20-minute admin day

The point is not to use AI more. It is to stop doing the same four tasks from scratch every evening.

When	What you do	Prompts	Time
After the last patient	De-identify and structure today's notes while the details are fresh	01, 04, 53	6 min
After the last patient	Draft tomorrow's new-patient programme and the WhatsApp version	16, 17	5 min
After the last patient	Send the after-first-session check and the day-before reminders	S3, S4	2 min
After the last patient	Log the day's numbers in one line (sessions, no-shows, enquiries)	54 (weekly)	2 min
Twice a week	One marketing asset: a post, a review request, or a Google post	59 to 64	5 min

11.2 The weekly hour

Pick one hour a week that is not a clinical hour. Sunday evening works for most solo practices. In that hour, run three things: the schedule audit (prompt 54), the WhatsApp audit (prompt 44) on ten messages you sent that week, and one growth task from Part 8. Write the two changes you will make next week in a notebook - not in your head.

11.3 The monthly half-day

- Numbers: run prompt 78 with this month's and last month's figures. One page, four numbers, one change.
- Data: run the retention and deletion routine (prompt 58). Ten minutes, and it keeps you honest about what you are holding.
- Content: batch four weeks of posts in one sitting (prompt 63), so the month does not depend on your mood.
- Team: if you employ anyone, the 15-minute check-in from prompt 75. If you do not, the capacity plan (prompt 80), once a quarter.
- One prompt to rewrite: take the prompt you used most this month and improve one block of it. That is how the pack becomes yours.

PART 12

12

The 30-day adoption plan

Fifteen minutes a day for thirty days, and the practice runs differently by December.

Do not try all eighty prompts in week one. You will use four of them badly and conclude that AI does not work for physiotherapy. Follow the sequence.

Days 1 to 3: the rails

- ☐ Day 1 - Read Part 1 and Part 2. Run prompt 01 on one old case note. Check the output has no name, number, address or date.
- ☐ Day 2 - Run prompt 02 on a brief you wrote yourself. Note what it caught.
- ☐ Day 3 - Set up your tool. Pick one paid tier or stay on free. Save prompts 01, 16 and 53 as notes on your phone.

Days 4 to 10: the evening

- ☐ Day 4 - Prompt 16 on one real patient. Compare the output to your current handout. Fix one thing in your handout permanently.
- ☐ Day 5 - Prompt 17 and 18 on the same patient. Send the WhatsApp version instead of the PDF.
- ☐ Day 6 - Prompt 53. Build the note template. Use it for every patient from today.
- ☐ Day 7 - Prompt 45 or 57. Fix one broken step in your intake.
- ☐ Day 8 - Prompt 46. Write your four reminder messages as saved replies.
- ☐ Day 9 - Prompt 50. Send one package balance update. Note the reply.
- ☐ Day 10 - Prompt 21. Set the seven-day adherence sequence for two patients.

Days 11 to 20: the money conversation

- ☐ Day 11 - Prompt 31. Write your fee-and-plan reply once. Save it as a saved reply.
- ☐ Day 12 - Prompt 32. Build your three tiers. Check each one is clinically deliverable.
- ☐ Day 13 - Prompt 33. Rehearse the two objections out loud. Pick the reply that sounds like you.
- ☐ Day 14 - Prompt 35. Write your 'pain is gone, problem is not' explanation. Use it once this week.
- ☐ Day 15 - Prompt 40. Write the insurance answer. Keep it in your saved replies.
- ☐ Day 16 - Prompt 36. Hand a discharge summary to one finishing patient.
- ☐ Day 17 - Prompt 30. Use the three-layer explanation in one consultation. Watch the patient's face.
- ☐ Day 18 - Prompt 43 or 44. Audit ten of your own messages.
- ☐ Day 19 - Prompt 59. Rewrite your Google Business Profile description.
- ☐ Day 20 - Prompt 60. Ask two finished patients for a review.

Days 21 to 30: the growth engine

- ☐ Day 21 - Prompt 62. Write one area landing page.
- ☐ Day 22 - Prompt 63. Build the 30-day content calendar. Shoot four clips in one sitting.
- ☐ Day 23 - Prompt 66. Write your referral letter. Deliver it to one doctor in person.
- ☐ Day 24 - Prompt 67. Send one apartment society proposal.
- ☐ Day 25 - Prompt 69. Run the reactivation to a small, selected list.
- ☐ Day 26 - Prompt 71. Publish your FAQ block.
- ☐ Day 27 - Prompt 54 and 78. Do the first full numbers review.

- ☐ Day 28 - Prompt 58. Do the first data retention routine.
- ☐ Day 29 - Prompt 80. Plan what leaves your hands next quarter.
- ☐ Day 30 - Go back to Part 14. Mark the twelve prompts you actually used. That is your pack now.

The honest expectation

This will not double your practice in a month. What it will do, if you follow the sequence, is return roughly an hour a day of admin, make your patient communication consistent, and give you a written record of why you made each clinical and business decision. That is a fair return on thirty days of fifteen-minute habits.

PART 13

13

Twelve ways AI makes a physio practice worse

The failure modes, in the order they actually happen, with the fix for each.

The mistake	What it looks like	The fix
1. Pasting identifiable data	A full name in a case brief because it was easier	Prompt 01, every time, until it is automatic
2. Accepting the first fluent answer	Output reads well, so it goes to the patient unchanged	Read it once out loud. If a sentence could apply to any patient, cut it
3. Letting it invent a dosage	A table with '3 sets of 12' that you never decided	Every prompt marks load as [CHECK]. Set it yourself, in your own hand
4. Letting it invent a citation	A guideline name or a study that does not exist	Ask for [VERIFY] instead of a source, then check the real one yourself
5. Sending unedited output	Patients receive a message that sounds like a brochure	Always edit the first two lines into your own voice. The opening is what patients judge
6. Over-relying on it for reasoning	Your hypothesis becomes whatever the model said first	Write your hypothesis before you open the tool. Then ask it to argue against you
7. Losing the record	Notes now live in a chat window, not in the patient's file	Prompt 53 exists so the record stays in your system, not in the model
8. Replacing the human call	A difficult conversation becomes a WhatsApp template	Template the routine, never the difficult. Bad news is spoken
9. Broadcasting to everyone	The same message to every patient, and the personal touch disappears	One variable per message: the patient's goal, their area, their name
10. Ignoring the compliance clock	No registration number on bills, no consent for WhatsApp, no data routine	Prompts 41, 51, 52 and 58 close this in an afternoon
11. Believing the productivity story	Three tools, six tabs, no time saved	One tool, twelve saved prompts, one routine. That is the whole stack
12. Never measuring	It feels faster, so it must be faster	Prompt 54 weekly. If the numbers do not move in six weeks, the workflow is wrong

The verification checklist, before anything reaches a patient

- ☐ No name, phone number, address, date, ID number or identifiable detail in what I pasted
- ☐ Every number in the output is either mine or marked [VERIFY]
- ☐ No diagnosis, prognosis or clearance that I did not make myself
- ☐ No claim about insurance, cure, timelines or success rates
- ☐ Language level right for this patient - and the language itself right
- ☐ My registration number and clinic details are correct where they appear
- ☐ I would be comfortable if this patient, their family and my council read it

PART 14

14

The prompt index

All eighty prompts and seven scripts in one place. Print this page.

No.	Prompt	Part
01	De-identify a case note before you paste it	02
02	Leak check: audit what you are about to paste	02
03	The prompt fixer - upgrade any request you already type	03
04	Turn a messy note into a structured subjective history	04
05	Build my hypothesis list - and argue against each one	04
06	Choose the right outcome measure for this patient	04
07	The red-flag screen I must not skip	04
08	Shortlist special tests I can actually perform here	04
09	A baseline objective record I can repeat at every visit	04
10	Reassessment: is this progress, a plateau, or a wrong plan?	04
11	Discharge readiness check	04
12	Home-visit feasibility and environment check	04
13	Adapt the same assessment for an older or a younger patient	04
14	The referral note a doctor will actually read	04
15	The 60-second case summary for a teleconsultation	04
16	A four-week programme with dosage and progression rules	05
17	The WhatsApp version - short enough to be read	05
18	The Hinglish version your patients will actually read	05
19	The one-page print handout	05
20	Progression and regression rules, written down once	05
21	A seven-day adherence check-in sequence	05
22	The patient says 'it hurts more after exercise'	05
23	No equipment? Substitute without losing the point	05
24	The bed-and-chair programme for a frail or post-operative patient	05
25	Return-to-sport and return-to-work criteria	05
26	The desk-worker micro-break plan	05
27	The caregiver training sheet	05
28	Explain a post-operative protocol without rewriting it	05
29	The programme in Hindi, in Devanagari	05
30	Explain the problem in three layers	06
31	Answer 'how many sessions and how much will it cost?' honestly	06
32	Three-tier package proposal	06
33	Objection: 'it is too expensive' / 'I will do the exercises at home'	06
34	The patient arrives with a doctor's note: '5 sittings only'	06
35	Why continue when the pain is already gone	06
36	The discharge summary a patient keeps	06
37	Brief the family without briefing them against the patient	06
38	The first phone call for a home-visit enquiry	06
39	The patient is already seeing another physiotherapist	06
40	The insurance answer that protects you later	06
41	Explain your registration and credentials, simply	06
42	The informed-consent conversation, said out loud	06

No.	Prompt	Part
43	Service recovery: reply to an unhappy patient	06
44	Audit my own WhatsApp replies	06
45	The intake form that does not waste the first session	07
46	The appointment confirmation and reminder sequence	07
47	No-show recovery that does not shame the patient	07
48	Cancellation and refund policy, in language patients accept	07
49	Home-visit route and slot planning logic	07
50	The package balance message	07
51	The itemised bill note for a patient who wants to claim	07
52	Consent form draft for treatment and for data	07
53	The clinical note template that takes 90 seconds	07
54	The weekly schedule audit	07
55	Consumables and equipment reorder list	07
56	The home-visit safety SOP	07
57	New patient onboarding SOP	07
58	Data retention and deletion routine	07
59	Google Business Profile that ranks for your area	08
60	Ask for a review without making it awkward	08
61	Reply to every kind of review	08
62	The area landing page that brings local patients	08
63	A 30-day content calendar you will actually keep	08
64	Three reel scripts that do not need a studio	08
65	Before/after posts without breaking trust	08
66	The letter that gets you doctor referrals	08
67	The apartment society camp proposal	08
68	The corporate ergonomics pitch	08
69	The reactivation campaign for past patients	08
70	A seasonal offer that is clinically honest	08
71	The website FAQ block that answers the real questions	08
72	The testimonial request with consent built in	08
73	The job description an associate physio will answer	09
74	Interview questions that predict how they treat patients	09
75	The first 30 days for a new hire	09
76	Train the assistant who is not a physiotherapist	09
77	Price your sessions with a number you can defend	09
78	The monthly numbers review	09
79	Break-even before you open a second location	09
80	The capacity plan for the year you stop doing everything	09
S1	New enquiry, first reply	10
S2	Appointment confirmation	10
S3	The day-before reminder	10
S4	After the first session	10

No.	Prompt	Part
S5	Package balance update	10
S6	Discharge message	10
S7	Reactivation, sent once a season	10

A last page, for the wall

Four sentences that hold this whole pack together.

> **The practice rule**
> I assess. The model drafts. I sign. The patient decides.

Everything else in these pages is technique. That one line is the discipline - and it is the reason a physiotherapist with a phone and a free AI account can now run the admin of a practice that used to need a back office, without handing over the part that cannot be handed over.

About this edition

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